

8 June 2023

Lucy Letby - Cross-examination Day 8: Babies O, P & Q
Housekeeping

CONTENTS

Housekeeping

MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

Housekeeping

Thursday, 8 June 2023
(10.30 am)

(In the absence of the jury)

Housekeeping

MR MYERS: My Lord, just before we proceed, can I raise one matter with your Lordship? We're grateful to the staff caring for Ms Letby for passing on to us, via Ms Clancy in the first instance, that Ms Letby has not been feeling well.

MR JUSTICE GOSS: She says she's not been feeling well.

MR MYERS: She says she's not been feeling well, hasn't eaten, but wants to continue, as I understand the position. That's what she would like to do. Naturally, in all the circumstances, that message causes concern to us, that, at the very least, we and the court are satisfied that she is in a proper state to continue.

And I don't know whether it's a matter of which your Lordship needs to make any enquiry. Obviously we cannot make any enquiry in the circumstances. But in any event, it's something we need to bring to your Lordship's attention so it's watched carefully.

We consider it would be helpful to have it confirmed with the witness, Ms Letby, that she is able to continue having received the message that we have, and that's for her benefit and everyone's benefit.

MR JUSTICE GOSS: Yes, exactly. I will do that.

MR MYERS: Thank you, my Lord.

MR JUSTICE GOSS: I gather you're not feeling very well?

LETBY: No.

MR JUSTICE GOSS: Right. Do you feel well enough to continue?

LETBY: Yes.

MR JUSTICE GOSS: Right. We will continue then. Should there come a time when you genuinely feel you are not fit to continue, would you please indicate and we will then pause the proceedings?

MR MYERS: Thank you, my Lord. May we raise concerns if we see anything that concerns us with your Lordship?

MR JUSTICE GOSS: Yes, certainly. It's a difficult balance to strike --

MR MYERS: It is.

MR JUSTICE GOSS: -- but the process must continue, but obviously not if she is unfit to give evidence.

MR MYERS: My Lord, yes.

MR JUSTICE GOSS: Yes, thank you.

(In the presence of the jury)

MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

MR JUSTICE GOSS: Mr Johnson.

MR JOHNSON: My Lord.

We're going to move on, please, to count 20, which concerns [Baby O]. As before, I'd like to start with the contents of your defence statement, so if you just want to put that thing away. Page 21, I hope, will take you to paragraph 168.

It reads as follows:

"I did nothing to hurt [Baby O]."

169:

"On 23 June 2016 I was allocated [Baby O], [Baby P] and another baby called J. Rebecca Morgan, the student nurse who was with me, fed [Baby O] at 8.30, 10.30 and 12.30. I signed the notes for this but she fed him down the NG tube and did his observations. Shortly after the 12.30 feed, [Baby O] vomited, he

didn't look right, and his observations were yellow on the charts."

And I assume that by yellow, you're referring to the -- in the yellow entry on the chart.

LETBY: Yes.

JOHNSON: 171:

"It had not been a profound vomit, however because [Baby O] didn't look right I got [Dr A] to review him, this will have been around 13.50, and nil by mouth was entered in the medical notes. At that point, there was a change in his appearance but it was not dramatic. [Baby O] remained in nursery 2."

So, so far, is what I have read to you what appears in the document?

LETBY: Yes.

JOHNSON: And is it correct?

LETBY: Yes.

JOHNSON: 172:

"At 14.40 [Baby O]'s monitors went off. He was not breathing well and his tummy was red and distended. I'd done nothing to cause this to happen. I called [Dr A] from nursery 3 and he used the Neopuff. He checked the blood gas, which was not good, and so we moved [Baby O] to nursery 1. Too many babies in there and so we had to move some out [as read]."

Again, is that what it says?

LETBY: Yes.

JOHNSON: And is it correct?

LETBY: Yes.

JOHNSON: 173:

"When [Baby O] collapsed people were panicking. The unit was very busy. We had to move another baby to make space for [Baby O] in nursery 1."

Is that what it says?

LETBY: Yes.

JOHNSON: Is it correct?

LETBY: Yes.

JOHNSON: 174:

"When [Baby O] was put on the ventilator in nursery 1, he stabilised but then deteriorated rapidly."

Again, is that what it says?

LETBY: Yes.

JOHNSON: And is it correct?

LETBY: Yes.

JOHNSON: 175:

"I didn't notice a rash to the right side of [Baby O]'s chest. No one mentioned such a rash. His hands and feet were pale and his stomach was red and purpuric and getting bigger all the time. At one point Dr Brearey put a needle into his stomach to see if anything could be drained. My recollection is that his stomach stayed red the whole time. I was present when we were resuscitating [Baby O] and it did not go."

So again, is that what it says?

LETBY: Yes.

JOHNSON: And is it correct?

LETBY: Yes.

JOHNSON: 176:

"[Baby O]'s abdomen was very swollen and the doctors were struggling to get access to a vein. One of [Baby O]'s lines had tissued and we were struggling to get lines into him. His peripheries turned white. He was so poorly it couldn't be done and the only option was to inject his shin and put a line into his leg."

Is that what it says?

LETBY: Yes.

JOHNSON: And is that correct?

LETBY: Yes.

JOHNSON: Are you there suggesting that intravenous access was lost?

LETBY: I'm saying that one of his lines had tissued, yes. One of the lines.

JOHNSON: Are you saying that intravenous access was lost?

LETBY: I can't say. From that I've read one of [Baby O]'s lines had tissued.

JOHNSON: You know the significance of intravenous access in this case, don't you?

LETBY: No.

JOHNSON: Well, we'll come to it. 177:

"[Dr B] kept going outside to smoke during the collapse. I appreciate it may seem a small matter in all the circumstances, but she didn't wash her hands when she returned."

Is that what it says?

LETBY: Yes.

JOHNSON: And is that true?

LETBY: Yes.

JOHNSON: Then finally:

"Sophie Ellis had raised a concern about [Baby O] and the appearance of his abdomen during the night shift before my shift commenced. I do not accept that this was dealt with adequately at the time."

Is that what it says?

LETBY: Yes.

JOHNSON: And is that true?

LETBY: Yes.

JOHNSON: And is that your position, that you do not accept it was dealt with adequately at the time?

LETBY: Yes.

JOHNSON: When did you come to that conclusion?

LETBY: By the time this statement had been written.

JOHNSON: What was it that caused you to reach that conclusion?

LETBY: Reviewing all of the medical records.

JOHNSON: So that is some time at or before, I think it was, 11 February that this document was signed off, 2022?

LETBY: Yes.

JOHNSON: Yes. That date is on the final page if anyone wants to check it.

All right. One thing I've overlooked is the question I ask you usually at the beginning of every day or every session, namely: up to this point in your evidence, is there anything that you want to amend or change or put a different emphasis on?

LETBY: No.

JOHNSON: So taking yesterday as the most recent example, is there anything you said yesterday that you now wish to review?

LETBY: No.

JOHNSON: Anything you felt you couldn't answer yesterday that, having thought about it overnight, you now feel that perhaps you can answer?

LETBY: No.

JOHNSON: Have you given that some thought?

LETBY: Yes, I've thought about it overnight, yes.

JOHNSON: Yes, you have thought about what you said yesterday and whether or not in fact you made a mistake; is that right?

LETBY: Yes.

JOHNSON: I want to deal with the background to [Baby O]'s death, please. We know from the evidence that we have heard that he was born on Tuesday, 21 June at 14.24. He died on Thursday, 23 June at 17.47. All right?

LETBY: Yes.

JOHNSON: At the time of [Baby O]'s birth, you were abroad, weren't you?

LETBY: Yes.

JOHNSON: You were with [Nurse E] and others in Ibiza?

LETBY: Yes.

JOHNSON: You had gone there on 20 June and you returned on the 22nd?

LETBY: Yes.

JOHNSON: And you were very interested in the triplets, even before you had seen them, weren't you?

LETBY: I received information about the triplets, yes.

JOHNSON: Would it be fair to say that they were big news in the unit?

LETBY: Yes, it was very unusual to have triplets on the unit, yes.

JOHNSON: Naturally conceived triplets. Had you ever in your experience come across naturally conceived triplets?

LETBY: No.

JOHNSON: If Mr Murphy would help, please, and we start at tile 21, this is your friend Jennifer Jones-Key telling you about their arrival; is that right?

LETBY: Yes.

JOHNSON: In the next text you make it clear that, as a matter of fact, you already knew.

LETBY: Yes.

JOHNSON: Is that right?

LETBY: Yes.

JOHNSON: Then we have tile 24 where you say:

"Probably be back in with a bang."

LETBY: Yes.

JOHNSON: Within 72 hours of that text, the two [Surname of Babies O, P & R] boys were dead and [Baby Q] had collapsed; that's right, isn't it?

LETBY: Yes.

JOHNSON: And all happened when you were there?

LETBY: Yes.

JOHNSON: Yes. You then texted your friend [Dr A] at tile 62, please -- I say text, it's a Facebook message -- still on the 22nd, where you were asking him for further detail about the triplets.

LETBY: Yes.

JOHNSON: Any reason why you were so interested?

LETBY: It was general conversation. Triplets were unusual.

Jennifer had obviously text me to say they were born, as had [Dr A]. It was just general conversation about something that's quite unusual.

JOHNSON: Did you want to get involved with them?

LETBY: No, I'm just making enquiries about something that's unusual on the unit that other members of staff are talking to me about.

JOHNSON: Do you accept the evidence of the boys' mother, [Mother of Babies O, P & R], that all went well at their birth?

LETBY: Yes.

JOHNSON: Do you accept that overnight, the day they were born, the 21st into 22 June, Amy Davies told us that there were no concerns for either [Baby O] or [Baby P]?

LETBY: Yes.

JOHNSON: Do you accept the evidence of Samantha O'Brien from the day shift on 22 June, so the day that you were sending the texts we've just looked at, when she said that [Baby O] in particular was fine on that day shift?

LETBY: Yes.

JOHNSON: And Yvonne Farmer, who said he was moved into nursery 2 because he was doing so well on that shift?

LETBY: Yes.

JOHNSON: Do you accept that overnight, from the 22nd into 23 June, both [Baby O] and [Baby P] were clinically well?

LETBY: Is that on this, on the 22nd?

JOHNSON: This is the day before -- this is the night shift before you came in on the 23rd.

LETBY: Yes.

JOHNSON: And do you accept that they were moved from CPAP to Optiflow on that night shift?

LETBY: I'd have to check. I can't recall that from memory, no.

JOHNSON: We'll come to the charts. Chris Booth told us that his impression was that [Baby O] would "sail through". Until [Baby O] collapsed, did you see anything to undermine that as a reasonable impression?

LETBY: No, not up until that lunchtime, no.

JOHNSON: Sophie Ellis actually looked after [Baby O] in particular that night and said that there was nothing concerning about his presentation; do you accept that?

LETBY: Yes.

JOHNSON: If you go, please, to jury bundle 2, which is, in your case, the one with the big blue 2 on the spine, and go to divider 20, please, you will find the charts and other documentation for [Baby O].

If you look at the second page, which is J23657, there we have the observation chart for [Baby O] from about the time of his birth; do you see that?

LETBY: Yes.

JOHNSON: And thus the clock went from the 21st to the 22nd, about halfway across the page, where we get to midnight.

Do you see that?

LETBY: Yes.

JOHNSON: It ran to 13.00 hours on the 22nd at the far right of the page.

LETBY: Yes.

JOHNSON: Then over the page, on page 23658, we have a chart starting at 14.00 hours on 22 June and moving through to the early hours and then ultimately the time of death of [Baby O]'s death on 23 June.

LETBY: Yes.

JOHNSON: In answer to questions asked by your counsel, I just want to confirm that you identified your handwriting on that chart, in particular where we see what looks like 13.30; is that right?

LETBY: Yes.

JOHNSON: Is that a time that's been changed as a matter of interest?

LETBY: It looks like I've gone over it in some way, yes.

JOHNSON: It looks like there was a 4 there at some stage, doesn't it?

LETBY: I can't -- I can't say for definite.

JOHNSON: Well, it's your writing. What does it look like to you?

LETBY: 13.30.

MR JUSTICE GOSS: You might be better looking at the screen in fact because it's enlarged on the screen.

LETBY: I would say it's gone from a 2 to a 3.

MR JOHNSON: Right. So no 4 under there?

LETBY: That -- that's my opinion, it's a 2 to a 4.

JOHNSON: Anyway, we know that problems for [Baby O] started, give or take, at about lunchtime, at about 13.15; all right?

LETBY: Yes.

JOHNSON: We'll come to it in the sequence, but looking at the readings, the observations, is there anything concerning, from your perspective, on that chart prior to that time that I have just drawn your attention to?

LETBY: His temperature's a little bit unstable at times.

JOHNSON: Yes, someone appears to have put a reading into the temperature chart at 12.30 and then crossed it out, don't they, and put a different one in?

LETBY: Yes.

JOHNSON: Was that you?

LETBY: No.

JOHNSON: How can you say?

LETBY: That's not how I would document a temperature. That's not my writing.

JOHNSON: That dot is not your writing?

LETBY: I don't believe so, no, because I join up -- I usually join up the arrows.

JOHNSON: Like you didn't do at 14.30 or 13.30?

LETBY: Because they're not a direct continuation. When there's two side by side, usually you should join them up, which is what's been done in the yellow bit. They actually join up.

MR JUSTICE GOSS: I think you'll have to keep your voice up, if you can, please.

MR JOHNSON: [Baby O], according to that chart, had been very stable overnight, hadn't he?

LETBY: He's had some temperature issues, but yes.

JOHNSON: Looking just -- if we can look at the bottom part of the page, please, Mr Murphy. That's it, thank you.

That left-hand column under where it says "Cooling infant", do you see it?

LETBY: Yes.

JOHNSON: It says "sats" and then "fractional oxygen".

LETBY: Yes.

JOHNSON: Then what does it say?

LETBY: "Optiflow."

JOHNSON: Yes. And this is the point I made to you earlier, that [Baby O] had been on Optiflow for quite some time, hadn't he, by the time --

LETBY: Yes. I couldn't recall that from my memory, so thank you for showing me.

JOHNSON: It's quite all right. It may be important, all right?

LETBY: Okay.

JOHNSON: His ECG leads had been removed and that would be in keeping with this picture, wouldn't it?

LETBY: Yes, if that's what happened, yes.

JOHNSON: That was the evidence.

LETBY: Okay.

JOHNSON: His antibiotics had been stopped and we know from tile 175, if Mr Murphy would help, please, that in a sample taken after his first collapse at 13.39, after 5 days there was no bacterial infection.

LETBY: No.

JOHNSON: If we then go, please, to tile 52, or in the paper version to 23958. An incorrect reference there. 23676.

No.

MR JUSTICE GOSS: No.

MR JOHNSON: That's my mistake.

MR JUSTICE GOSS: I don't think we have a paper version of either of those.

MR JOHNSON: It's 23661. It's my mistake in my note, my Lord. 23661.

MR JUSTICE GOSS: Four from the back.

MR JOHNSON: Yes.

If we could scroll down just a little bit, please, Mr Murphy. The aspirates in the output section under the NG/asp/vomit column show that [Baby O] was tolerating his feeds very well, don't they?

LETBY: Yes.

JOHNSON: Going back to your defence statement then, and what you wrote at paragraph 178, what are you saying is the point that you don't accept that the appearance of his abdomen overnight was dealt with adequately?

LETBY: I think concerns were raised about his abdomen and feeds were continued.

JOHNSON: But what was actually going on shows that there wasn't a problem, doesn't it?

LETBY: I -- I don't know.

JOHNSON: Well, you do know, you're a band 5 QIS qualified nurse. Where, looking at the data, is the problem?

LETBY: I -- I can't say without looking.

JOHNSON: Well, you take your time and look and you tell us where you are saying this problem was.

(Pause)

LETBY: He's still having milky aspirates.

JOHNSON: You give us the page that you're looking at, please.

LETBY: 23676.

JOHNSON: So if we carry on in the documents to the final document behind divider 20, what's the problem?

LETBY: There's not a problem, it's just that he is still having aspirates and he hasn't had his bowels open.

JOHNSON: Doesn't it say bowels opened, for example, at 5 o'clock, and he passed urine as well?

LETBY: I think that's a BNO.

JOHNSON: BNO?

LETBY: Yes.

JOHNSON: Well, you see, this is one of the reasons I went to your defence statement first because you raise an issue and I'll just remind you of what you said at paragraph 178:

"Sophie Ellis raised a concern about [Baby O] and the appearance of his abdomen during the night shift. I do not accept that this was dealt with adequately."

What is the inadequacy?

LETBY: I don't know.

JOHNSON: You don't know?

LETBY: No.

JOHNSON: So you are suggesting that there was a problem but you can't -- you're not in a position to tell us what the problem was?

LETBY: Mm.

JOHNSON: Is that what it comes to?

LETBY: Yes.

JOHNSON: And there's no apparent problem recorded in the paperwork; is that right?

LETBY: Yes, not on the charts, no.

JOHNSON: Do you accept Dr Mayberry's account of examining [Baby O] at the end of the night shift and that all was well?

LETBY: Is this the night when he reviewed him but didn't write the notes?

MR MYERS: Yes, it is.

LETBY: So yes, on his word, yes.

MR JOHNSON: Yes. Well, are you suggesting we shouldn't accept his word if he didn't write it down himself?

LETBY: No, I'm saying there's no way of definitively knowing if he reviewed him or not.

JOHNSON: Well, Sophie Ellis wrote down that he had because, as you remember, I'm sure, he had been called away and didn't have time to make the note himself.

LETBY: Yes.

JOHNSON: So when you say there's no way of definitively knowing, if he didn't examine him, would it follow you're suggesting

there must have been some sort of cover-up between Dr Mayberry and Sophie Ellis?

LETBY: No, I'm saying there was no formal note, so therefore I was not there, I cannot say definitively what happened.

JOHNSON: Right. Well, do you accept the nursing note that he did it?

LETBY: Yes.

JOHNSON: Right. Do you accept that Dr Mayberry essentially excluded the presence of the subcapsular haematomas in [Baby O]'s liver at the time of that examination?

LETBY: Yes.

JOHNSON: If we could go, please, to Sophie Ellis' note at tile 105. So:

"Abdo looks full, slightly loopy, appeared uncomfortable after feed. Registrar Mayberry reviewed. Abdo soft. Does not appear in any discomfort on examination. Has had bowels open."

So there's a note made by your colleague that his bowels had opened; do you agree?

LETBY: Yes.

JOHNSON: "To continue feed but to monitor."

Is that note, in effect, the foundation for your assertion in your defence statement that there was some sort of inadequate treatment?

LETBY: I think I've stated because the doctor didn't actually write anything down, I don't know that he was fully reviewed.

JOHNSON: Right. So because Dr Mayberry personally didn't make a note, you do not accept that any issue that there may have been with [Baby O] was adequately dealt with at the time?

LETBY: Yes.

JOHNSON: So it comes down to the personalisation of the note, that's the basis for your concern?

LETBY: Yes, because it's been pointed out where something's not written down, you don't know that it happened.

JOHNSON: And who's pointed that out?

LETBY: Sorry?

JOHNSON: You say it's been pointed out that; by who?

LETBY: You.

JOHNSON: Me? When's that?

LETBY: Well, people have stated that if something's not written down or somebody writes something, it might not be correct.

JOHNSON: I am sorry, it's probably me, but I don't really -- I want you to be able to tell the jury, to explain so that everyone understands, and I don't understand what you're saying.

MR MYERS: If I can help, it was the suggestion made in the case of Dr Beebe's afternoon review of [Baby I] when in the nursing note it was pointed out that Ms Letby had said there was a review at about 15.00 and the prosecution pointed out there was no note and that's what she is referring to, that it's been pointed out before.

MR JOHNSON: Is that what you meant to say?

LETBY: Yes, so therefore it didn't happen, yes.

JOHNSON: I'm very grateful to Mr Myers.

Do you accept that at 09.30 Dr Cooke examined [Baby O] and actually felt the liver area and excluded there being an injury at that point?

LETBY: Yes.

JOHNSON: So does it therefore follow that the liver injury [Baby O] did sustain happened, in effect, on your watch?

LETBY: Yes.

JOHNSON: Yes. Do you accept, therefore, that somebody must have inflicted that liver injury on [Baby O] during that day shift?

LETBY: I agree that it's happened during that shift. I don't know how it's happened.

JOHNSON: Well, we've heard expert evidence from Dr Marnerides about how these things happen, haven't we?

LETBY: Yes.

JOHNSON: Yes. Your case is you don't know how it's happened and I assume you will say that you deny that it happened at your hand?

LETBY: That's right.

JOHNSON: Well, let's go to the population distribution, please, at tile 106. There we see that you were the only nurse in nursery 2, is that right --

LETBY: Yes.

JOHNSON: -- with [Baby P], [Baby O] and a child JD?

LETBY: Yes.

JOHNSON: The third triplet, [Baby R], was in nursery 1 with [Baby Q], BM and YM. And the nurses in there were Samantha O'Brien and Christopher Booth. Is that right?

LETBY: Yes.

JOHNSON: Three children in nursery 3: ZZ, MB and FG, with Mel Taylor and [Nurse C]?

LETBY: Yes.

JOHNSON: Then another couple in nursery 4 with [Nurse C]; is that right?

LETBY: Yes.

JOHNSON: So it follows that you were the only nurse in nursery 2 on this shift?

LETBY: Yes.

JOHNSON: Are you suggesting that the staffing levels caused or contributed to [Baby O]'s collapse or death?

LETBY: No.

JOHNSON: Are you suggesting that any doctor's mistake caused or contributed to his collapse or death?

LETBY: No.

JOHNSON: Are you suggesting that any nursing mistake caused or contributed to [Baby O]'s collapse or death?

LETBY: No.

JOHNSON: You, of course, were supervising, was it Rebecca Morgan --

LETBY: Yes.

JOHNSON: -- the student on this shift? But she wasn't always in the nursery with you, was she?

LETBY: No.

JOHNSON: She was off doing things with other people and having a chat to some of the mums?

LETBY: Yes.

JOHNSON: We can see you telling your friend [Dr A] about that at tile 149 at 10.20 that morning, can't we?

LETBY: Yes.

JOHNSON: One of the things you said -- well, let's deal with it this way: is the fact that you had three children in nursery 2 a reflection of the fact that they were all doing very well?

LETBY: Yes, it would suggest that they're not all high dependency.

JOHNSON: Was [Baby O] high dependency?

LETBY: I don't believe so, no.

JOHNSON: No. It's just in your evidence on 17 May you suggested that he was.

LETBY: Well, I'm not sure without clarifying with the guidelines. I can't remember.

JOHNSON: Or was it that on the 17th you were trying to create the impression that things were rather more fraught and difficult than really they were?

LETBY: No, I can't recall now what access he had and what would make him ITU or non-ITU.

JOHNSON: Do you agree that high dependency babies should never be left unattended?

LETBY: No, they are left unattended but somebody would be aware that you're leaving the room, yes.

JOHNSON: So someone would keep an eye out?

LETBY: If you're going to be away for a period of time, yes.

JOHNSON: And what do you mean by "a period of time"?

LETBY: So if I was leaving the nursery to, say, leave the unit then I would get somebody to listen out. If I was going to get medicines from another nursery then I wouldn't necessarily do that because I would still be able to hear the monitors.

JOHNSON: Okay. I want to start with tile 121, please. These are Dr Cooke's notes of an examination that she undertook of [Baby O] at 09.30.

LETBY: Yes.

JOHNSON: Dr Cooke told us that the designated nurse, which we know was you, told her that there were no concerns for [Baby O]; do you accept that?

LETBY: Yes.

JOHNSON: She told us that reported to her were trace of aspirates but none bilious; do you accept that?

LETBY: That I told her that?

JOHNSON: Yes.

LETBY: I don't recall saying that, but there weren't any -- there's no bile on this chart, no.

JOHNSON: No. Well, you were either completing the chart or supervising the student completing the chart, weren't you?

LETBY: At this point there is only one feed, but yes.

JOHNSON: Yes. Dr Cooke said there was absolutely nothing abnormal with [Baby O] at this stage of the morning; do you accept that?

LETBY: Yes.

JOHNSON: And she positively excluded the possibility of there having been an abdominal issue at that point; do you accept that?

LETBY: Yes.

JOHNSON: Did you leave the unit shortly after this?

LETBY: I've left the unit at points, yes. I think I went to go and get some donor milk out of the fridge -- freezer, sorry.

JOHNSON: Where is the freezer?

LETBY: It's on the corridor outside of the neonatal unit doors.

JOHNSON: I'm just interested in a sequence of text messages that you then started to send to your friend, [Dr A].

All right? Can we start at 123 on the basis that you're the only nurse in nursery 2 and, if you're going off the unit, you get someone to listen out.

You say "Boo" at 123. Is that trying to get his attention?

LETBY: I think it's in response to a previous text where he said he's not on the unit, I think.

JOHNSON: Yes. Is that because -- yes, you're quite right.

LETBY: He text me first and I think this is my response to him not being on the neonatal unit that day.

JOHNSON: He was in the clinic, I think, it's tile 119.

Were you disappointed that he wasn't there in the unit?

LETBY: Yes, I enjoyed working with [Dr A], yes.

JOHNSON: Did you want to get his attention?

LETBY: No.

JOHNSON: Is that the reason you sabotaged [Baby O]?

LETBY: No.

JOHNSON: If we go to 124 next, please, Mr Murphy. In answer to "Boo" he says:

"I thought something similar."

Then 3 minutes later, 125, you sent another message.

Then added a correction to that at 126.

At 127:

"Are you here this aft?"

And he replied:

"Yes, back after the clinic."

At 128. So at that stage you knew he would be back in the afternoon, didn't you?

LETBY: Yes.

JOHNSON: You then said:

"Have fun."

130:

"My student is glued to me."

He commiserated saying:

"Could be a challenge."

Then:

"This is what my morning is looking like."

I say "my" because of the next text.

You responded:

"How exciting."

He said:

"No, it's a load of..."

And you said:

"Now, now, [Dr A]."

Then at 09.45:

"Bit rubbish that you couldn't stay on NNU."

Were you missing him?

LETBY: No, this was my first day back at work.

JOHNSON: At 9.47, 138:

"It's okay. Tony would have had a whole day of clinic otherwise. Clearly fridge sales may be low."

And you then sent two further messages about the Eskimos at 139 and then about whether or not he would get lunch.

And your texts continued at 142, 09.55. You said you'd lost your handover sheet and found it in the donor milk freezer. He replied and you continued texting him on and off between 10.00 hours and 10.39. Was that a busy morning for you?

LETBY: Sorry?

JOHNSON: Was that a busy morning?

LETBY: It was a fairly busy shift, yes, not exceptionally busy.

JOHNSON: Given that you don't text in the clinical areas, you have told us, where do you get the time from to be sending all these texts?

LETBY: A lot of the staff are often on their phones.

JOHNSON: No, no, I'm not asking about --

LETBY: When you're not doing something directly with the baby, I could be on my phone.

JOHNSON: In the clinical area?

LETBY: I wouldn't be at the cot side, no, but I might be on the unit using my phone, yes.

MR JUSTICE GOSS: Sorry, "on the unit" --

LETBY: (Overspeaking).

MR JUSTICE GOSS: -- would you be in the nursery, in this case nursery 2, when you were making or sending --

LETBY: No, I wouldn't have used the phone in the clinical area.

A non-clinical area would be somewhere like the nurses' station, the equipment room, the break room, the corridor. Anywhere that's not at the cot side.

MR JOHNSON: At tile 158, please, 10.36, [Dr A] was telling you - - he was offering to get you your lunch and saying that he thought clinic should be done in an hour, so in other words, by about 11.35; is that right?

LETBY: Yes.

JOHNSON: And when he arrived, we know he came to see one of [Babies O, P & R] because, if we look in the -- just put that file to one side for a second and find the black neonatal reviews. It's event 21 and 22, [Baby O] and [Baby P], so it's the third from the back, the sixth in. Go to page 3. I think I said one of [Babies O, P & R], I made a mistake. It's actually [Baby Q]. It's line 35; do you see that?

LETBY: Yes.

JOHNSON: So that shows, doesn't it, that [Dr A] was on the neonatal unit at 12.10?

LETBY: Yes.

JOHNSON: At 12.30 you fed [Baby O], didn't you?

LETBY: Could I just see the chart, please?

JOHNSON: It's tile 167, please, Mr Murphy.

In the hard copies it's 23676. So if anyone's still got jury bundle 2 out, it's behind divider 20, it's page 23676.

That shows you feeding [Baby O], doesn't it?

LETBY: No, that's not my writing. I signed the bottom, but the actual feed content is not my writing.

JOHNSON: Whose initials are at the bottom?

LETBY: Mine.

JOHNSON: Yes. So you fed [Baby O]?

LETBY: No, it's not, no. That's not my writing, that's Rebecca's writing, and as her mentor I have to countersign that she's done anything.

JOHNSON: Yes, but she should sign it as well, shouldn't she, if she did it?

LETBY: Or if I've done it first she wouldn't need to, no.

JOHNSON: If what, sorry?

LETBY: If I've signed as well then she doesn't need to, necessarily, no.

JOHNSON: Why is that?

LETBY: Because if she's fed back to me that she's fed the baby, I would take her word for that and document it myself, the initials.

JOHNSON: There were no problems at that feed according to the data that's recorded on the form, were there?

LETBY: No.

JOHNSON: Trace aspirate?

LETBY: Mm-hm.

JOHNSON: And your initials?

LETBY: Yes.

JOHNSON: And it was at about this time that you deliberately overfed [Baby O], wasn't it?

LETBY: No, I did not overfeed [Baby O] and I did not feed him this feed.

JOHNSON: Do you remember that it was around about this time that Mel Taylor was in nursery 2?

LETBY: Yes.

JOHNSON: If we look at line 57 of the neonatal review, we can see that at 13.00 hours she made an entry of having fed the child JD.

LETBY: Yes.

JOHNSON: And JD was one of your children in nursery 2, wasn't he?

LETBY: Yes.

JOHNSON: So Mel was in there, in nursery 2, at about -- in the aftermath of your record that you had fed [Baby O]; is that right?

LETBY: Yes, so Mel was with baby JD at 1 o'clock, yes.

JOHNSON: The point is she's in nursery 2; is that right?

LETBY: Yes.

JOHNSON: And you know the significance of Mel being in nursery 2 at that time, don't you?

LETBY: Because she says she made comment about [Baby O] not looking well.

JOHNSON: Yes. She said to you that [Baby O] didn't look well.

What else did she suggest?

LETBY: From her recollection that we should move him into nursery 1.

JOHNSON: Yes. And you refused, insistentlly, didn't you?

LETBY: I don't recall the details of the conversation, no.

JOHNSON: Well, do you recall a different conversation?

LETBY: I don't recall any specific details about talking about going into nursery 1. I know that nursery 1 was very full. I don't remember being very dismissive to Mel about where [Baby O] needed to be.

JOHNSON: She told the jury that you were undermining her authority.

LETBY: That's Mel's opinion if that's what happened.

JOHNSON: Mm. Well, I'm going to suggest to you, from what we saw yesterday with [Baby N], you weren't getting on with Mel at this point, were you?

LETBY: Yes, I got on with all my colleagues, professionally.

JOHNSON: "FFS", do you remember that; yes?

LETBY: Yes.

JOHNSON: You were angry with Mel; you accepted that.

LETBY: Yes.

JOHNSON: Was it your place to refuse if a senior colleague made a suggestion?

LETBY: No, I don't recall doing that, but if I had, Mel would have been perfectly within her rights to override me and take the baby off me.

JOHNSON: But she didn't?

LETBY: No.

JOHNSON: You had sabotaged [Baby O], hadn't you?

LETBY: No.

JOHNSON: And that's why you didn't want him moving outside your control into nursery 1?

LETBY: No.

JOHNSON: Because, as we can see from the population distribution, if a child went from 2 to 1, it would necessarily involve a child coming out of 1 and in all likelihood being swapped into 2?

LETBY: Yes.

JOHNSON: Keeping you in 2?

LETBY: I can't answer that. That would be up to how the shift leader would have allocated.

JOHNSON: Well, it would, but two of your charges remained in nursery 2?

LETBY: Yes.

JOHNSON: And this would have meant [Baby O] escaping your influence, wouldn't it?

LETBY: No, I disagree. I can't say how the allocation would have worked had [Baby O] moved.

JOHNSON: We've looked just now, and it may be that the jury still have open chart 23676 behind divider 20, which shows a trace aspirate at 12.30. Do you agree?

LETBY: I'll have to get it back out.

JOHNSON: That's all right.

LETBY: Yes.

MR JUSTICE GOSS: It's the last page in section 20. Do you know what tile it's behind? I haven't got a note of it.

MR JOHNSON: I do, yes: it's tile 167.

Why did you lie to [Dr A] about this aspirate?

LETBY: I don't think I did lie to him.

JOHNSON: Well, let's look at tile 168. Let's look at his original note, please. It may be my mistake, we'll see.

Is that "no bile"?

LETBY: Yes.

JOHNSON: Sorry, that is mistake, I apologise for that. I read that as "bile".

All right. So that's an examination by [Dr A] in the aftermath of what I'm going to suggest was a collapse for [Baby O] at 13.15.

LETBY: Okay.

JOHNSON: You would have been giving this information to your friend [Dr A], wouldn't you?

LETBY: Yes, most likely.

JOHNSON: It gives the amount of milk that [Baby O] was receiving, a vomit post-feed, no bleed, small meconium plug, heart rate 160 to 170, et cetera, doesn't it?

LETBY: Yes.

JOHNSON: 160 to 170 isn't tachycardic, is it?

LETBY: It's higher than we would want it -- well, it's the higher end of normal.

JOHNSON: Yes. Well, if we go in the paper bundle to 23656, we see that the yellow area starts at 180, doesn't it?

LETBY: Yes.

JOHNSON: If we look at your nursing note at tile 169, we see that you were recording at this examination that [Baby Q] was tachycardic. That wasn't an accurate reflection --

LETBY: You've just said [Baby Q].

MR JUSTICE GOSS: [Baby O].

MR JOHNSON: Sorry, I said [Baby Q]. [Baby O]. [Baby O] was tachycardic.

LETBY: Yes.

JOHNSON: Is that an accurate reflection in reality?

LETBY: Yes. He's had an increasing heart rate over the period of time and when I got him reviewed it suddenly spiked up further, so...

JOHNSON: He was not tachycardic, was he?

LETBY: I think he was in my nursing opinion.

JOHNSON: All right, in your nursing opinion?

LETBY: Yes.

JOHNSON: I see. Do you remember saying on 17 May that this was not a concerning vomit, it was a standard baby vomit?

LETBY: Yes.

JOHNSON: Do you remember the lying false entry you made in a different chart at about this time?

LETBY: No.

JOHNSON: Let's look at tile 170, please. It's J23667. Right at the back -- one from the back of the paper documents behind divider 20. You see there your writing at 13.20?

LETBY: Yes.

JOHNSON: What's the false entry in that line?

LETBY: I don't know.

JOHNSON: You don't know? Do you remember Dr Bohin telling us about it?

LETBY: No.

JOHNSON: CPAP?

LETBY: Yes.

JOHNSON: Do you remember now?

LETBY: I think he had some CPAP pressure at that point, did he not?

JOHNSON: No. He'd not been on CPAP for hours and hours. You were covering for air you'd given him, weren't you?

LETBY: No.

JOHNSON: Let's look at tile 197. This is an X-ray that was taken at about the time of [Baby Q]'s second collapse at about 14.40. Okay? Sorry, [Baby O]; I keep saying [Baby Q] for some reason, I'm sorry. Round about [Baby O]'s second collapse at about 14.40; okay?

LETBY: Okay.

JOHNSON: Do you remember Professor Arthurs looking at this and telling us about it?

LETBY: Yes, I know he looked at it, yes.

JOHNSON: Yes, and he said that the level of gas in the bowel was more than would be expected in a normal baby. He gave two possibilities as being the reason: one was NEC and the other was air down the nasogastric tube. Do you remember that?

LETBY: Yes.

JOHNSON: That's the reason you wrote CPAP in the gas chart, isn't it, so that anyone looking back might think: well, if he was on CPAP that might account for why he's got gas in his bowel?

LETBY: No.

JOHNSON: Well, why did you write CPAP in the gas chart?

LETBY: I can't answer that now, I don't know.

JOHNSON: Were you upset that he wasn't on CPAP?

LETBY: He wasn't on the full CPAP machine -- I can't say whether he was having CPAP via the Neopuff. I don't know.

JOHNSON: That's a medical decision, isn't it?

LETBY: Yes.

JOHNSON: It's not for a nurse to put a child on CPAP?

LETBY: No.

JOHNSON: No. Going back to the observation chart, which is 23658, no mention of CPAP in there, is there?

LETBY: No.

JOHNSON: Optiflow, as we saw before?

LETBY: Yes.

JOHNSON: Yes. Did you forget to make a false entry in that chart as well?

LETBY: No.

JOHNSON: Do you agree that [Dr A] left the neonatal unit after the review of [Baby O]?

LETBY: If that's what he says, yes. I don't have any recollection. Is this the first collapse or the second?

JOHNSON: That's the first collapse at about 13.15.

LETBY: I know he was back on the unit by the time of the second event.

JOHNSON: Yes, well, we're going to come to that in a moment. One of the reasons I'm suggesting he left, if we look, is to be found at tile 177 -- 178, I beg your pardon. My eyesight.

You were texting him?

LETBY: Yes.

JOHNSON: And you wouldn't have needed to be texting him if he was there in the unit, would you?

LETBY: Yes.

JOHNSON: So can we take it that he was off the unit?

LETBY: Yes.

JOHNSON: If we go to 183, please, he was replying to you. 184.

185. So your conversation, your text conversation, carried on, didn't it?

LETBY: I -- looking at this now, I disagree that [Dr A] wasn't on the unit. This is [Dr A] referring to cannulating [Baby O] and I'm not present.

JOHNSON: So you think you were not on the unit or what?

LETBY: [Dr A] was -- can we just go back to the --

JOHNSON: Yes, please.

LETBY: -- previous one? And again. So this is [Dr A] saying to me that he's got a cannula in on [Baby O] and, sorry, he may have dripped a little, meaning blood, on his sheet.

This is [Dr A] informing me that that's what he's done.

I'm not sure whether I was on a break or... But this is [Dr A] telling me what he has done and I'm telling him to leave and go and get his lunch.

JOHNSON: Right, okay. 190, please. It suggests that he had had his lunch.

LETBY: Yes.

JOHNSON: You think you were on a break, do you, between 14.08 and 14.37?

LETBY: I'm -- I don't know where I was, but [Dr A] is texting me, so I'm assuming I was not in the nursery at that time.

As we said, he would have been able to say it verbally if I was there.

JOHNSON: Well, if we look at tile 187 we see that as those messages were going backwards and forwards, when you say you were on a break and he was in the unit, you were actually taking observations of [Baby O] at 14.30.

LETBY: Yes.

JOHNSON: So you were on the unit?

LETBY: Is this exchange before or after?

JOHNSON: Well, we've been through -- I'll go through them again if you want to. All right? The start of the messaging started at 14.07.

LETBY: Right.

JOHNSON: We then have 14.08, 14.15 -- sorry, that's [Nurse E]. I'll start again to make sure I don't make a mistake.

The first message is at 14.08, which is tile 178.

The next message from him to you, 183 at 14.20. Then from you to him at 14.23, 184.

MR JUSTICE GOSS: I think, to interrupt you, that's 14.20, 14.23. That is when the witness is saying that she is likely to have been away and he was in the unit.

MR JOHNSON: Yes, thank you.

We see that you are taking observations at 14.30, don't we?

LETBY: Yes. But I think this exchange took place before that.

JOHNSON: And we can also see, if we look at the neonatal review document at line 91, you were making nursing notes in the case of JD at 14.30.

LETBY: Yes.

JOHNSON: And it was shortly after that and shortly after the departure of [Dr A] that [Baby O] collapsed again, didn't he --

LETBY: Yes.

JOHNSON: -- at 14.40?

LETBY: Yes.

JOHNSON: Can you remember what you told the jury about this collapse when you were answering questions from your own counsel?

LETBY: Not in great detail, no.

JOHNSON: Well, I'll remind you. You said that you heard the monitor alarming. When you went into the nursery, you found it was [Baby O] and you called for [Dr A], who was in a nursery next door.

LETBY: Yes.

JOHNSON: So he'd come back by this stage, had he?

LETBY: Yes, I recall that [Dr A] was on the unit at the time, yes.

JOHNSON: I think you then suggested that you weren't on the unit at all by reference to tile 193, if you remember that.

If we look at tile 193. This is door swipe data. It shows you going from the labour ward into the neonatal unit.

LETBY: Right.

JOHNSON: I think you told us yesterday that there's a doorway there, I think this was when you came in and saw [Baby N] on 3 June at 7.12. Do you remember? We had the same sort of data.

LETBY: Yes. I've entered the unit via the maternity ward, yes.

JOHNSON: So where were you at around this time?

LETBY: I can't say definitively, but I've come through the labour ward.

JOHNSON: Had you just nipped out to make it look like you weren't actually around when [Baby O] collapsed?

LETBY: No.

JOHNSON: One of the things we see in the X-ray report that I showed you before -- it's 197, please, Mr Murphy -- is that the NG tube was in a satisfactory position.

Do you see that?

LETBY: Yes.

JOHNSON: At 199 we have [Dr A]'s note. Just to remind you of what he said, he refers to being called to see [Baby O] at about 14.40. Was it you that called him?

LETBY: From my memory, yes.

JOHNSON: Were you trying to get his attention?

LETBY: Yes, I wanted him to review [Baby O], yes.

JOHNSON: But his personal attention as well for you?

LETBY: No. [Dr A] was the registrar on the unit that day.

JOHNSON: If we look at tile 200, please. What we see there is some of your handwriting; is that right?

LETBY: In the second column, yes.

JOHNSON: Yes. So it's the second row; is that right?

LETBY: Yes.

JOHNSON: "23 June, saline 0.8, 20ml, bolus, intravenous."

Prescribed by Dr Cooke much earlier that morning, wasn't it, at about 9.30?

LETBY: I'm not sure what time she prescribed that.

JOHNSON: But not given until this time. Any reason for that?

LETBY: How do you know it was prescribed earlier?

JOHNSON: It's what -- I think Dr Bohin referred to it.

LETBY: Well, I can't answer that. There's no way, looking at that sheet, to say when it was prescribed.

JOHNSON: You record in the nursing note at tile 201 -- I've been asked...

MR MYERS: It seemed a suitable point and I just wonder if this is the point to take a break. It's been an hour, my Lord.

MR JUSTICE GOSS: Yes. We'll have our 15-minute, or thereabouts, break now, please, members of the jury.

(11.45 am)

(A short break)

(12.00 pm)

MR JOHNSON: It may be that I have introduced a level of confusion about something which I want to resolve if we possibly can. All right?

I want to start by going -- we need to look at several documents, please. The first is tile 200, the one we were looking at before.

If we just look at this, please. Do you remember me showing you this before?

LETBY: Yes.

JOHNSON: So in the second row of data you have co-signed with somebody else; is it Samantha O'Brien?

LETBY: Yes.

JOHNSON: For giving a saline bolus, we can see --

LETBY: Yes.

JOHNSON: -- of 20ml. The time you've put -- that's your writing in the date and time, isn't it?

LETBY: Yes.

JOHNSON: The time you've put in there is 14.40. I suggested it was prescribed by Dr Cooke because we see Dr Cooke's signature in the column immediately to the left of your signature.

LETBY: Yes.

JOHNSON: I think where I fell into error was attributing that event, the prescription that is, to the 9.30 examination, whereas in fact, if we go to [Dr A]'s note, which we looked at earlier at tile 168 -- it may be easier to stay on this page rather than go to his slightly difficult-to-read handwriting. We see just above where it says "observations":

"Plan 1: gas 2 if temperature instability/apnoea/bradycardia. PO/PR aspirated. Blood. Abdominal X-ray. IV fluids: 10ml/kg bolus."

And then abdominal X-ray.

LETBY: Yes.

JOHNSON: Is that right?

LETBY: Yes.

JOHNSON: So 10ml per kilo. We know that when he was born, [Baby O] was about -- well, he was 2.02 kilos, so even my maths can work out that's a 20ml bolus.

LETBY: Yes.

JOHNSON: So it appears that it was [Dr A] that in effect made the order, but Dr Cooke who wrote the script.

LETBY: Yes.

JOHNSON: Would that fit?

LETBY: Yes.

JOHNSON: Would you agree that the reason, as you understood it as the designated nurse, for the order from [Dr A] was this was a child that needed a bolus of saline because he'd just vomited?

LETBY: I don't think the saline was in response to the vomit, I think it was his poor blood gas.

JOHNSON: All right. Whatever the reason, there was a fairly important need for it?

LETBY: Yes.

JOHNSON: So the question, please, is why you delayed giving the bolus for such a long time?

LETBY: He needed to have access obtained. He didn't have a peripheral line, I don't believe, at this point.

JOHNSON: Right. Well, who put the peripheral line in?

LETBY: I think [Dr A] did.

JOHNSON: Is that the reference in the text?

LETBY: Yes.

JOHNSON: I see. And so it follows, if we then go to [Dr A]'s -- so you'd been on a break when he was doing that, is that what you're saying, you left the child?

LETBY: Potentially. I don't know that I was definitely on a break but I was not with [Baby O] because [Dr A] was texting me.

JOHNSON: "Potentially" is a word you use rather a lot.

LETBY: Because I can't say 100% that I was on my break at that point. There's no way of ever being able to know what time somebody's break was definitively.

JOHNSON: No, but if a child had desaturated significantly, sufficiently to require the attendance of [Dr A], who you appear to have been looking forward to seeing, would you have chosen that moment, as he was treating and putting a line into the child, to go on your break?

LETBY: Yes, breaks are allocated by the shift leader so I would have had to have been guided by them.

JOHNSON: Anyway, the fact is that you gave the bolus to [Baby O] just before he collapsed, didn't you?

LETBY: No, I believe that the bolus was given as a result of the collapse.

JOHNSON: Well, the interesting -- we may never get to the bottom of this, but if we go to tile 199, it was something that [Dr A] was asked about, it says at the bottom, you can see -- again, we'll rely on the typed version for now, after the word "cefotaxime" it says:

"... plus 10ml/kg 0.9 sodium chloride bolus already given."

LETBY: Yes.

JOHNSON: You're saying that's a different bolus?

LETBY: I'm saying the bolus that I've documented is the 14.40.

That's the only bolus that I've written anything against.

JOHNSON: Yes. But I'm asking about the coincidence between the second collapse of [Baby O] and you belatedly, for the reasons you have described, administering this bolus of sodium chloride.

LETBY: Again, I don't know what time that was prescribed, whether it was for the previous sodium chloride bolus or whether it was for the 10% that needs to be given now at 14.40.

JOHNSON: Well, who of the nursing staff has principal responsibility for [Baby O] at this stage?

LETBY: Me.

JOHNSON: And this second desaturation was very significant, wasn't it?

LETBY: Yes. More so than the first one, yes.

JOHNSON: Profound; would you agree?

LETBY: Yes.

JOHNSON: That was the word you used at tile 201. Go there, please. You record:

"Profound desaturation to 30s followed by bradycardia and mottled ++. Abdomen red and distended. Transferred to nursery 1 and Neopuff ventilation commenced. Perfusion poor."

This was a life-threatening event, wasn't it, that level of desaturation?

LETBY: Yes.

JOHNSON: Before the events that the jury have been hearing about during the course of this trial, which events started at the beginning of June 2015, had you seen lots of desaturations like these?

LETBY: Yes, frequently babies desaturate, yes.

JOHNSON: But to this level?

LETBY: Yes, they can do.

JOHNSON: I know they can do. I'm asking you whether it happens very often.

LETBY: Yes.

JOHNSON: Do you remember what you said about the mottling, as you have described it -- or "mottled" as you've described it in that nursing note -- what you said about that in interview?

LETBY: No.

JOHNSON: I'll just remind you. It's in the second interview bundle, please, and it's the first of the [Baby O] interviews. So the [Initials of Baby O] section.

LETBY: Which page, sorry?

JOHNSON: [Initials of Baby O] 7. Sorry, you're struggling to hear? [Initials of Baby O] 7, please.

My fault.

It's below the second hole on the page. You say:

"So I remember it well. He was mottled over his -- he was mottled all over and he had a red abdomen. So the mottling is a sort of blotchy purply-red rash and a red abdomen as well."

Is that how you remembered it?

LETBY: Yes.

JOHNSON: Had you seen that sort of mottling before?

LETBY: Yes, mottling is common.

JOHNSON: This isn't normal mottling, is it?

LETBY: The redness to his abdomen was abnormal, yes.

JOHNSON: The mottling is abnormal as well, isn't it, that you're describing there?

LETBY: No, I would say that's my description of mottling.

JOHNSON: So serious was this event that [Baby O] had to be intubated, didn't he? Do you remember?

LETBY: Yes.

JOHNSON: And that happened at 15.03 that day. Do you remember what Dr Brearey saw when this was going on?

LETBY: The intubation?

JOHNSON: Yes.

LETBY: No.

JOHNSON: You really don't remember what Dr Brearey saw on [Baby O]?

LETBY: No. I cannot remember what every witness has said with every baby, no.

JOHNSON: I don't doubt that but I'm going to suggest you do remember very well what Dr Brearey saw. I'll remind you of what he said. He said:

"I noted during the intubation that [Baby O] had a rash on his chest, on the right side of his chest wall, exactly 1 or 2 centimetres in size. I haven't said the size there..."

And he's referring to the note:

"... but I mentioned it in my statement. It was an unusual rash. It was initially -- it initially appeared like a purpuric rash, so it was a little concerning and clearly noticeable."

That, if anyone wants to check, is evidence he gave on 15 March at page 29 of the transcript.

He went on to say that the rash disappeared later on. Do you remember now that that was what he said?

LETBY: Yes.

JOHNSON: And that is what you saw as well, isn't it?

LETBY: No.

JOHNSON: Did you have to think about that?

LETBY: Yes.

JOHNSON: Do you agree that that's what was there?

LETBY: Again, I don't believe that's what I saw, I saw mottling, but if that's what Dr Brearey saw...

JOHNSON: "If that's what Dr Brearey saw", what was the end of the sentence, please?

LETBY: If that's what Dr Brearey saw and that's what people take as being true, then yes.

JOHNSON: Do you remember Dr Brearey telling us that at this stage [Baby O] had a mild, non-metabolic acidosis?

LETBY: Okay, yes.

JOHNSON: And that that was explicable by blood loss?

LETBY: Yes.

JOHNSON: Do you agree with that?

LETBY: I don't think as a nurse that I can comment on what has caused a blood level result.

JOHNSON: All right. Do you remember that when [Dr A] had intubated [Baby O], he went to see [Baby O]'s parents on the maternity ward?

LETBY: Yes.

JOHNSON: And when [Dr A] disappeared, [Baby O] declined again, didn't he?

LETBY: Yes.

JOHNSON: And at 15.49, he, [Dr A], and [Dr B] were together on the stairs. Do you remember them saying they met on the stairs by chance --

LETBY: Yes.

JOHNSON: -- and the bleeps, their bleeps, went off? Do you remember that?

LETBY: Yes.

JOHNSON: Just to remind us, please, Mr Murphy, these are tiles 214 to 217. We can just scroll through the carousel, thank you.

There's the entry of [Dr B] back into the neonatal unit; okay?

LETBY: Yes.

JOHNSON: This was [Baby O]'s final decline, wasn't it?

LETBY: Yes.

JOHNSON: Do you remember it happening?

LETBY: This specific moment where I've put a crash call out?

JOHNSON: Yes.

LETBY: No.

JOHNSON: Do you remember it?

LETBY: No.

JOHNSON: It was a very dramatic death, wasn't it?

LETBY: Yes, I remember his death, I don't remember this exact moment that he declined.

JOHNSON: Do you remember how you felt when this child, who had already collapsed twice, collapsed for a third time just after your friend had gone to see his parents?

LETBY: Yes, I was concerned for him.

JOHNSON: But you don't remember it?

LETBY: Not with any detail, no. I remember the death, but not this precise moment that he desaturated and needed a crash call, no.

JOHNSON: Do you remember this was another case of the doctors establishing good air entry into [Baby O]'s lungs and yet he was, that is [Baby O], desaturating?

LETBY: Yes.

JOHNSON: He was intubated again; do you remember that?

LETBY: Yes.

JOHNSON: He was given CPR and two doses of adrenaline; do you remember that?

LETBY: Yes.

JOHNSON: And do you remember that his spontaneous circulation was restored?

LETBY: Yes.

JOHNSON: And the rash that had been there earlier had disappeared; do you remember that?

LETBY: I don't recall the rash disappearing, no.

JOHNSON: You see, Dr Brearey and [Dr B] both said that this sequence is something they had never seen before. Had you seen that sort of thing before?

LETBY: I didn't see what they saw. He had a red and distended abdomen.

JOHNSON: When did you pull the nasogastric tube out of [Baby O]'s stomach?

LETBY: I don't recall pulling a tube out of his stomach.

JOHNSON: Let's look at tile 246, please. This was an X-ray taken to establish the position of the ET tube; do you see that?

LETBY: Yes.

JOHNSON: It says:

"ET tube in situ in satisfactory position. NG tube [so the nasogastric tube] in situ with its tip close to the cardia [it says]."

Got that?

LETBY: Yes.

JOHNSON: "It should be advanced [in other words put in] by 10 to 15 millimetres", is what it says.

LETBY: Yes.

JOHNSON: I showed you an X-ray report earlier that was taken at about the time of the second collapse, 14.40 --

LETBY: Yes.

JOHNSON: -- in which Dr Wright reported that the NG tube was in a good position.

LETBY: Yes.

JOHNSON: In terms of a child with a distended abdomen who may have had air in his stomach, what effect would pulling the nasogastric tube back have?

LETBY: I believe it would still drain because it's still in the stomach.

JOHNSON: It's not in a good position, is it?

LETBY: No, it needs to be advanced, yes.

JOHNSON: Yes, to be effective it needs to be further in; do you agree?

LETBY: I don't know that the tube is not working at that position. I agree that it needs to be advanced, yes.

JOHNSON: How would a -- in a child who's intubated, how would the tube be -- the NG tube come out?

LETBY: During the intubation, the taping that's on the baby's face securing the tube would need to be taken off to allow for the NG tube to be put in and secured, so potentially it slipped a little bit while that's been doing -- being done.

JOHNSON: Did you take that as an opportunity to make sure that the tube wasn't draining [Baby O]'s stomach --

LETBY: No.

JOHNSON: -- all as part of your efforts to kill him --

LETBY: No.

JOHNSON: -- by pumping him with air?

I'm going to ask you about descriptions given by [Mother of Babies O, P & R] and [Father of Babies O, P & R]. All right? And like many of the other parents' evidence, this evidence was agreed. All right? So I'm going to ask you whether you still agree it.

Do you remember [Mother of Babies O, P & R] saying that [Baby O] was:

"... changing colour with discolouration and prominent veins."

And that she saw something similar with her other son, [Baby P] -- I should say one of her other sons, [Baby P], the next day. Do you agree with that description?

LETBY: I didn't see that. I didn't see that myself.

JOHNSON: [Father of Babies O, P & R], do you remember [Father of Babies O, P & R]?

LETBY: Yes.

JOHNSON: Do you remember his video --

LETBY: Yes.

JOHNSON: -- played to the jury? What he said was this:

"You could see [talking of his son] all his veins, bright, bright blue, going different colours. Looked like he had prickly heat. You could see something oozing through his veins."

Do you agree with that?

LETBY: I didn't see anything like that, no.

JOHNSON: So we've got Dr Brearey with the disappearing rash and [Mother of Babies O, P & R] and [Father of Babies O, P & R], and [Father of Babies O, P & R] talking about a moving pattern of colour.

LETBY: Yes.

JOHNSON: That's the truth, isn't it?

LETBY: I can't comment on their truth. I did not see anything like that myself.

JOHNSON: What you saw was a sort of blotchy purply-red rash and a red abdomen?

LETBY: Yes.

JOHNSON: Do you remember you were asked by your counsel about the CPR that was given to [Baby O]?

LETBY: Yes.

JOHNSON: And do you remember what you said about it?

LETBY: No.

JOHNSON: You said it was done by others but not by you.

LETBY: Okay.

JOHNSON: Do you remember?

LETBY: I don't remember, no.

JOHNSON: You don't remember saying that? Well, let's see what you said to the police if we can, please, at [Initials of Baby O] 9, so the same interview that we had before. This is the very end of what we have of the interview of 5 July 2018. The police asked you:

"Okay, so, 'CPR commenced at 16.19. Medications/fluid given as documented'. What was your role in his CPR?"

You said:

"I think I did some chest compressions and I think I did some drugs, but I'd have to check."

Right?

LETBY: Yes.

JOHNSON: What's changed your mind since that interview?

LETBY: I've been able to look at the charts to see that I was doing the medications.

JOHNSON: Yes. What's changed your mind about the CPR?

LETBY: Because if I was doing the medications at those times, I would not have been able to be carrying out compressions.

JOHNSON: Well, why didn't you say that here then?

LETBY: Because at this point in interview, I didn't have any details of seeing what I had done during the resuscitation.

JOHNSON: You said:

"I think I did chest compressions and I think I did some drugs."

LETBY: Which is standard practice, yes.

JOHNSON: Yes. Nurses rotate, don't they, on these occasions?

LETBY: Yes.

JOHNSON: Particularly with prolonged resuscitations?

LETBY: Yes.

JOHNSON: Because it's important to keep the CPR going at the right rate?

LETBY: Yes, so it'd be -- many staff would be involved, yes.

JOHNSON: Yes. I'm going to suggest a reason why you have -- you are now saying it wasn't you doing CPR, because you don't want to have any connection between you and the liver injury because you're now running a case that it was the CPR that caused the liver injury.

LETBY: No.

JOHNSON: You don't agree with that?

LETBY: No.

JOHNSON: Of course when you gave this interview, this was well before Dr Marnerides ever wrote a report, wasn't it?

LETBY: Yes.

JOHNSON: Yes. How did [Baby O] get that liver injury?

LETBY: I don't know.

JOHNSON: Do you accept the evidence of Dr Marnerides?

LETBY: Yes.

JOHNSON: Because he also told us that [Baby O] didn't have any bowel disorder; do you remember that?

LETBY: Yes.

JOHNSON: So no bowel disorder to account for all the gas in his bowel. Professor Arthurs told us that if a bowel disorder was excluded we're left with air going down the NGT to account for the gas in the bowel; do you accept that?

LETBY: If that's what he said, yes.

JOHNSON: That's why you made the false entry about CPAP, isn't it --

LETBY: No.

JOHNSON: -- to account for the gas?

LETBY: No.

JOHNSON: You injected [Baby O]'s stomach with gas down the NGT, didn't you?

LETBY: No, I did not.

JOHNSON: You injected air into his circulation, causing an air embolus?

LETBY: No.

JOHNSON: And you, through some violent mechanism, inflicted that liver injury on him, didn't you?

LETBY: No.

JOHNSON: These things all happened on your watch, didn't they?

LETBY: Yes.

JOHNSON: Do you dispute that Dr Brearey tried to persuade you not to come in after [Baby O]'s death?

LETBY: Yes.

JOHNSON: Why did you want to come in after that?

LETBY: I didn't feel, if that's what Dr Brearey had offered, that I needed time off.

JOHNSON: Were you not bothered by what you'd witnessed?

LETBY: Of course I was bothered, yes.

JOHNSON: I would like to deal with a few texts that you sent in the aftermath of [Baby O]'s death, please, starting with tile 331.

It might be better to do 330, actually, because you need to know what [Dr A] said to deal with this.

So this was at 21.13, an hour and 13 minutes after your shift had ended. He was sending you a message saying:

"Your notes must have taken a long time. Had you documented anything from this morning?"

And you responded:

"Only a little. Had the other two to write on as well and sorting out the FFP..."

Is that a blood transfusion?

LETBY: Yes, fresh frozen plasma.

JOHNSON: Fresh frozen plasma:

"... et cetera. Left signing for drugs until tomorrow."

Which drugs are you referring to as being signed the next day?

LETBY: Um, I couldn't say definitively. I'm assuming any of the drugs that were given that afternoon.

JOHNSON: Are they all -- I'm not going to take you through them one by one because I can do this some other time, but are these all drugs that are, when they're signed, they are electronically stamped with a time signature by the computer?

LETBY: No, it could be computerised records or it could be any paper drugs as well. I couldn't say which drugs exactly I'm referring to.

JOHNSON: So we're not going to be any the wiser by looking at the records, are we?

LETBY: No, because I'm not sure whether it means signing electronically or signing by hand on a paper form.

JOHNSON: Okay. The notes for [Baby P], as a matter of record, and we can come to this in due course, if we go to tile 23 on the [Baby P] sequence, took you about 3 minutes to write, between 20.24 and 20.27. We'll look at that in due course.

I want to just move on, though, to 334, please.

This is a conversation that you were having with [Nurse E], who had asked you what had happened about a minute earlier. Just for the full picture, actually, if we go back to the previous one. In fact, let's go back to 332 and have the whole conversation, please.

There she is asking you what happened in graphic terms at 332. You replied:

"Can't think straight so took a while."

And then the next one, please:

"Blew up abdomen. Think it's sepsis."

Is that a phrase you use often, "blew up abdomen"?

LETBY: No.

JOHNSON: How many cases of "blew up abdomen" have you seen?

LETBY: I've seen a few over my time.

JOHNSON: I'm going to suggest that you were also telling your friend [Nurse E] lies in these texts. Okay? So if we go to 342, please. Would you just read that, please?

LETBY: "Had big tummy overnight but just ballooned after lunch and went from there."

JOHNSON: That wasn't really an accurate statement of what had been happening with [Baby O] the night before he died, was it?

LETBY: I believe he had had an enlarged abdomen overnight.

JOHNSON: This is one of the reasons why at the beginning we went through what had happened on the night shift.

LETBY: Yes.

JOHNSON: There was nothing wrong with him when you took him over in the morning, was there?

LETBY: He'd just been reviewed by the doctor for having a loopy bowel.

JOHNSON: Yes, but that's not "big tummy overnight", is it?

LETBY: I disagree. I think I am referring to some distension that was there prior to me taking over at 8 o'clock.

JOHNSON: Then tile 403, please. If you want the context I'll give it to you. We can look at it if you want to, all right? So if you don't want to take my word for it, by all means we can look at the texts. It begins with [Nurse E] at 386 saying:

"We don't have any luck with 33/34 weekers."

You agree and you may remember the police asked you questions about that in interview. And the conversation goes on, saying -- with [Nurse E] saying at 398:

"Hope other two have an easy ride now for the parents' sake."

That's a reference to the other two triplets, isn't it?

LETBY: Yes.

JOHNSON: And you say:

"Yeah, worry she's missed something."

Then you say:

"Home at last."

Which is -- that's to [Dr A] at tile 401. To [Nurse E] you say:

"Fingers crossed."

And then:

"Worry as identical."

So this is a text that you are sending at the same time or within the same minute as the text you were sending to [Dr A] saying that you'd just got home?

LETBY: Yes.

JOHNSON: Were you setting up a false narrative here? Do you understand what I mean by that?

LETBY: Yes, and no, I wasn't, no.

JOHNSON: You were trying to create in the minds of other people that because [Baby O] had died of some unspecified problem, there was a likelihood that the same thing was going to happen to one of his siblings.

LETBY: No, that's not what I'm suggesting at all.

JOHNSON: Because that's what you were planning, wasn't it?

LETBY: No, it was not.

JOHNSON: And you had already put into motion your plan by pumping air into [Baby P] before you left?

LETBY: No.

JOHNSON: Do you remember the Datix form that you put in relating to [Baby O]'s death?

LETBY: Yes, I think there was something in relation to equipment, was there?

JOHNSON: Well, we will come to this, all right? I'm trying to deal with things chronologically. We will come to a text message that you had stored on your phone that was never actually sent. It was in effect a note to yourself. Do you remember?

LETBY: Yes.

JOHNSON: It was on a date right at the end of June, reminding yourself to put in Datix forms; do you remember that?

LETBY: Yes.

JOHNSON: One of those Datix forms was the Datix form that you put in for [Baby O]; do you remember?

LETBY: Okay.

JOHNSON: We'll just look at it, please, it's at tile 497.

Can we go to the actual form, please, because in the sequence this is split over two tiles, but the form itself appears as a typewritten complete entity.

So what we see is that on 29 June 2016, at 08.36, this form was submitted.

LETBY: Yes.

JOHNSON: And I think you've already told us that you go into the child's records, there is a drop-down menu, you can select a Datix form, which then gives you various options that you fill in as you go along; is that right?

LETBY: Yes, you'd open up the Datix sort of app and it would produce you with this, with drop-down pieces throughout, yes.

JOHNSON: So you fill in the child's details; is that right?

LETBY: No, that I believe comes from the system.

JOHNSON: Okay. That's what I was trying to explain, but I obviously didn't do a good job. Never mind.

Let's scroll down, please. So you say "clinical incident" -- sorry, just slightly -- you've gone slightly too far.

So "clinical incident" is what you are suggesting; is that right?

LETBY: Have I filled in this form?

JOHNSON: Well, okay, let's get -- we'll establish that first.

Can we just scroll down, please, to the next page?

There we are. So "employees involved".

LETBY: Yes, the incident reporter though was Eirian Powell.

JOHNSON: I see. Eirian Powell, of course, wasn't on duty on this shift, was she?

LETBY: No, she wasn't.

JOHNSON: So where would she have got the information from?

LETBY: From any of the notes that were available, medical and nursing notes, word of mouth.

JOHNSON: Word of mouth?

LETBY: Well, the deaths would be discussed, yes.

JOHNSON: This was one of the things on your list to -- your note to self to submit, wasn't it?

LETBY: A Datix, yes.

JOHNSON: Yes. So you had a hand in this, didn't you?

LETBY: Not in this Datix, no. I believe it was about equipment that I was filling in the Datix for.

JOHNSON: Do you remember Dr Brearey telling us that the information in this Datix is untrue?

LETBY: In this one here?

JOHNSON: Yes.

LETBY: Yes.

JOHNSON: Do you remember what the untrue statement in the Datix is?

LETBY: He didn't believe at any point that IV access was lost.

JOHNSON: Yes, exactly. So Eirian Powell, if what you're saying is right, is making a report of an incident at which she was not present?

LETBY: Yes, because she's the ward manager.

JOHNSON: Yes. I'm suggesting to you that you in effect were giving her the information and that's why your name features as the employee involved.

LETBY: No, the employee involved would always be the designated nurse, so yes, it was me.

JOHNSON: Yes. If we scroll down, please, to what's actually been reported -- I'm just trying to find the narrative, sorry. It should have:

"Situation: one of the spontaneous triplets born died suddenly and unexpectedly."

Then it says "Incident reporter" somewhere. I'll find it on here and I'll get it right.

(Pause)

You've found it, thank you.

So there we have:

"Triplet 2 born."

And it gives his details. Is that right so far?

LETBY: Yes.

JOHNSON: "On 23 June at 13.15 he vomited undigested milk, became tachycardic."

So that's taken directly from your nursing notes, isn't it?

LETBY: Yes.

JOHNSON: You were the one who said he was tachycardic:

"He was reviewed by the registrar. Septic screen. 14.40, profound desaturation followed by bradycardia. Transferred to nursery 1. Put on Neopuff. Doctors crash called at 15.51..."

This is [Dr B] and [Dr A]; is that right?

LETBY: Yes.

JOHNSON: "... due to further desaturations down to 30. Chest movement and air entry."

We've just dealt with all that:

"Responded to treatment. Back on to the ventilator. 17.15 baptised. CPR following further desaturation. Six doses of adrenaline and other drugs. Decision made to stop. [Baby O] passed away at 17.47."

Right so far?

LETBY: Yes.

JOHNSON: Is that how you remember it?

LETBY: I don't recall writing this, no. I don't think I filled it in.

JOHNSON: Okay. Let's keep going if we can. Scroll down.

Further again, please. That's all behind the scenes information; is that right?

LETBY: Yes.

JOHNSON: Down again. A meeting, an incident review group; is that right?

LETBY: Yes.

JOHNSON: And again, please. And again, please. Keep going, please. Down to the bottom.

Dr Brearey said that the suggestion that IV access had been lost in the form was untrue; do you remember?

LETBY: Yes. Was that in the form there?

JOHNSON: Well, I'm having a problem locating it, I'm afraid.

(Pause)

LETBY: I think it just talks about "needle aspiration to abdomen and IO access obtained".

JOHNSON: Okay. I think I've made a mistake with the -- can we go to 498, please, Mr Murphy? I'm sorry, it's my mistake.

Let's go to the original, please. It's me getting confused between two reports back-to-back. Right.

LETBY: Yes, this is the form that I completed.

JOHNSON: Yes. Thank you. This is 30 June; is that right?

LETBY: Um, it's -- yeah, it's for the date of the 30th. It's completed on the 1st.

JOHNSON: Right. If we scroll down, please. So:

"Infant had a sudden acute collapse requiring resuscitation. Peripheral access lost. IO access required. Resources not available."

Right?

LETBY: Yes.

JOHNSON: Further down -- do you remember this, do you remember completing this?

LETBY: Oh, not this -- this isn't me completing this, but no.

I know I put the form in, yes, because we didn't have the IO access on the unit.

JOHNSON: Okay. Let's just carry on, please. So the patient details are [Baby O]'s details; yes?

LETBY: Yes.

JOHNSON: You are the incident reporter on this?

LETBY: Yes.

JOHNSON: Then the report further down, please.

(Pause)

Sorry, it's further up.

(Pause)

So:

"Infant had sudden acute collapse requiring resuscitation. Peripheral access lost."

That was not true, was it?

LETBY: I think at the time, yes, it was.

JOHNSON: No, no. Dr Brearey has told us it was not true.

LETBY: Okay. That's Dr Brearey's opinion.

JOHNSON: Well, it is Dr Brearey's opinion, you're quite right, but it's also what's in the medical records. How [Baby O] was being given some of the resuscitation drugs was through the peripheral access that was not lost. Do you agree?

LETBY: Yes.

JOHNSON: Yes. Thank you.

MR JUSTICE GOSS: If you go on to the next page, in fact.

MR JOHNSON: Yes, Dr Brearey actually...

MR JUSTICE GOSS: "Incident investigation: reviewed on 25 July."

MR JOHNSON: Yes:

"Patient did not lose peripheral access. IO access required for blood samples only."

LETBY: I accept that and the form was written with the regards (sic) of we did not have that equipment on the unit.

JOHNSON: No, but you understand the allegation here, or part of the allegation, is that you injected air into [Baby O]'s circulation, don't you?

LETBY: Yes.

JOHNSON: If there was no peripheral access, that would not be possible, would it?

LETBY: No.

JOHNSON: No. And that is the reason you were suggesting that when this child had his fatal collapse he did not have peripheral access.

LETBY: That's not what I was suggesting. I'm saying it was lost during resuscitation.

JOHNSON: That wasn't true, was it? Why were you making an untrue statement in a Datix form?

LETBY: I don't believe it was untrue at the time.

JOHNSON: Is it just a coincidence that you were making this report at pretty much the time that you were being removed from the unit?

LETBY: I hadn't been removed from the unit at this point.

JOHNSON: When did you leave the unit as you remember it?

LETBY: Some time in July.

JOHNSON: Right at the beginning of July.

LETBY: Okay.

JOHNSON: You were very worried that they were on to you, weren't you?

LETBY: No.

JOHNSON: Well, we'll come to the texts. Do you remember those panicked texts you were sending [Dr A] because Eirian Powell had phoned you up at short notice to cancel you coming in for a shift?

LETBY: Yvonne Griffiths, yes.

JOHNSON: So you do remember?

LETBY: Yes.

JOHNSON: And you were very worried they were on to you, weren't you?

LETBY: No. I was worried what was happening, yes.

JOHNSON: All right. We're going to move on to [Baby P], please. If I can I'll just deal with some of the background.

MR JUSTICE GOSS: Please.

MR JOHNSON: Thank you.

Can we look at the defence statement, please?

Count 21, please, ladies and gentlemen. Paragraph 179.

It begins with you denying having hurt [Baby P]. 180:

"In the evening of 23 June Belinda Simcock asked me if I wanted to look after [Baby P] on the 24th. I said I did. I wanted to do this as continuity for the benefit of the parents rather than them seeing me working with other babies after what had happened with [Baby O]. I don't recall having an argument with Kate Percival-Ward about wanting to be in nursery 1.

"On 24 June [Baby P] was in nursery 2. A little after 09.35 I was talking with Rebecca Morgan when [Baby P] stopped breathing."

Just to remind us, Rebecca Morgan is the student; is that right?

LETBY: Yes.

JOHNSON: "Dr Ukoh was in nursery 2 already because he was writing up his notes. The monitor sounded and I shouted for [Dr B] because she was already on the unit. I also called [Dr A] to assist."

At 183:

"[Dr B] came, a crash call went out. A lot of doctors and nurses came. It was chaotic. [Baby P] was too ill to move to nursery 1 so he remained in nursery 2. When Dr Rackham arrived, he took control of what was happening."

184:

"Events during the collapse and resuscitation are difficult to remember."

185:

"[Baby P]'s stomach was distended when he collapsed but it was not as red as [Baby O]'s had been. I don't recall [Baby P]'s skin being mottled."

186:

"After [Baby P] died, we talked about dressing the twins. I was asked to dress them in the same outfits. Christopher Booth and I did this in nursery 2. Sophie Ellis took photos of [Baby P] with [Baby O] and [Baby R]. There's nothing strange about this. It's part of the bereavement pathway. I did the bereavement pack and memory box."

187:

"At some stage during this, I pricked myself on a cannula needle which had been put into [Baby P]'s leg. I went to the accident

and emergency department as was standard procedure with a needlestick procedure and I was there for about an hour. Whilst there, I fainted and had to be given medical attention. I think this is because of the stress and upset of the past 2 days and because I hadn't eaten. I completed the bereavement procedure and the memory box after returning from A&E."

188:

"On 23 June 2016 I had been looking after [Baby P], [Baby O] and another baby. I kept the handover sheet from 23 June, so I would know what I was doing. I had written notes on the back of the handover sheet. I took it home to bring it into work the next day to write up the notes on the unit. The sheet stayed with me and after [Baby P] died I forgot about it and brought it home with me and it stayed there, although I had forgotten about it. I did not decide to keep it."

Do you accept that [Baby P] was born in good condition?

LETBY: Yes.

JOHNSON: The day after his birth, Wednesday, 22 June, do you accept that he was doing well and that at 11.45 that morning he was taken off CPAP by [Dr B]?

LETBY: Yes.

JOHNSON: Do you accept that, that day, his blood gases were excellent?

LETBY: Yes, I haven't reviewed them myself, but yes, I accept that.

JOHNSON: Thank you. On Thursday, 23 June, which was the day [Baby O] died, and if we could just look at tile 22, please, I'm going to suggest this shows that [Baby O] was taken off Optiflow at 6 am. If we scroll down, please.

Can you see?

LETBY: Yes.

JOHNSON: Thank you. You'll remember that this day or the time at which [Baby P] was taken off Optiflow became a subject of confusion when some of the witnesses were giving evidence; do you remember that?

LETBY: Yes.

JOHNSON: Dr Gibbs, for example, thought that he'd been on Optiflow much closer to the time of his death than in fact was the case.

LETBY: Yes.

JOHNSON: Yes. And Dr Gibbs therefore mistakenly attributed the distension of his stomach and bowel to Optiflow.

LETBY: Yes.

JOHNSON: Yes. But in fact, he wasn't on Optiflow at all for many hours prior to his death, was he?

LETBY: No.

JOHNSON: No. At 10 am, and this is tile 64, so we're still on the 23rd, the day [Baby O] died, the day you were in nursery 2 with [Baby O], [Baby P] and the other child, an examination by Drs Cooke and [Dr A] was unremarkable; do you agree?

LETBY: Yes.

JOHNSON: And do you accept that throughout that day there was nothing unusual at all so far as [Baby P] was concerned?

LETBY: Not in the daytime, no.

JOHNSON: No. Events, I suggest, started to take a turn for the worse just after [Baby O] had died, [Baby O] having died at 17.47; do you agree?

LETBY: Yes.

JOHNSON: Yes. Do you remember that Dr Gibbs examined [Baby P] in the aftermath, the immediate aftermath, of [Baby O]'s death?

LETBY: Yes.

JOHNSON: That's tile 133, please. This is you feeding [Baby P], isn't it, at 18.00 hours; do you remember?

LETBY: No, again, that's not my writing.

JOHNSON: Sorry?

LETBY: That's not my writing.

JOHNSON: What's not your writing?

LETBY: The 18.00 feed.

JOHNSON: Oh right. Whose signature is it at the bottom?

LETBY: Possibly mine.

JOHNSON: Possibly yours? Who else's might it be?

LETBY: I can't see for definite, but it looks like mine, yes, but that's not my writing.

JOHNSON: As a matter of interest, do you remember what you said in interview about this when you police asked you about it?

LETBY: No.

JOHNSON: No, well, we'll come to it. You think about it over the break.

Do you remember Dr Gibbs saying that when he examined [Baby P], he was remarkably well for a triplet baby and very well grown?

LETBY: No, but if that's what he said...

JOHNSON: He did. If we go to tile 145, please, we see that a blood test was also taken at about this time.

LETBY: Yes.

JOHNSON: And that came back as normal. Do you accept that?

I think the result may be further down.

LETBY: Yes, the culture is negative.

JOHNSON: Yes. Five days later, tragically several days after he died, of course, [Baby P]'s blood, at the time you were coming to the end of your shift on which we suggest you had murdered his brother, was entirely normal.

LETBY: From a blood culture perspective, yes.

JOHNSON: Yes. What other perspective would be relevant?

LETBY: So this is purely looking at if there's any bacterial growth within a venous blood source. It doesn't describe CRPs or any other blood issues.

JOHNSON: There weren't other any issues at this stage, were there?

LETBY: Okay.

JOHNSON: Were there?

LETBY: I can't say without looking, I've only seen this.

JOHNSON: But you've told me more than once that you do look at the papers in advance of giving evidence about these cases.

LETBY: Yes. I can't remember it all.

JOHNSON: No, no one would expect you to remember everything, but have you identified an issue that was ongoing at this time?

LETBY: No.

JOHNSON: With [Baby P], sorry.

LETBY: No.

JOHNSON: No. Thank you. You overfed [Baby P] some time between 18.00 and handing him over at 20.00, didn't you?

LETBY: No, I did not.

JOHNSON: We'll just deal with that point I asked you about earlier, about Rebecca Morgan and the feed at 18.00 hours, just before we finish. Go to the [Baby P] interviews, please, to the very first interview. Do you see there at [Initials of Baby P] 3 the police are asking about the feeding chart that we just looked at?

LETBY: Yes.

JOHNSON: Can you see that you say:

"At 10 o'clock he received a further feed via his NG tube..."

LETBY: Yes.

JOHNSON: You say:

"... by the student nurse, countersigned by myself. Then 12 o'clock, nappy care, small vomit. All done by the student nurse, co-signed by me. Another feed at 14.00 via the NG tube, done by the student nurse, co-signed by me. Another feed at 16.00..."

Just read what you say there then, please?

LETBY: "... via his NG tube and that was signed by the student nurse and then by myself. And there was a feed at 18.00 via his NG tube and that's signed by myself."

JOHNSON: Then:

"Is there a reason why Rebecca hasn't signed after 4 o'clock?"

What do you say?

LETBY: "Students usually only work an early shift so potentially she had gone home."

JOHNSON: "Potentially she'd gone home":

"Is there a reason why you've got entries for 4 and 6 o'clock?"

LETBY: "When a student does a feed they have to inform somebody that they've done it to get a co-signature. I have co-signed that one and then I believe I may have done this feed for somebody at 6 o'clock."

Can I point out that I did not have the chart at this point.

JOHNSON: "You've done it for somebody? Is that common practice then to sign on someone's behalf?"

And you say?

LETBY: "No. I mean -- I've done the feed."

JOHNSON: Yes.

LETBY: "But I've done it on behalf of somebody if they've had to leave."

JOHNSON: You say you didn't have the chart. Can you go to the bottom of the previous page, please? What's your reply at the bottom?

LETBY: "So this is the feed chart. He's been fed at 8 o'clock in the morning and that's via the student nurse and co-signed by myself."

JOHNSON: "So this is his feed chart here."

What did you mean by that?

LETBY: Sorry?

JOHNSON: "Okay, so this is his feed chart here."

What did you mean by that?

LETBY: That's his feed chart for the 23rd.

JOHNSON: Yes. You've just told the jury that you didn't have the feed chart.

LETBY: But I --

JOHNSON: There you are saying you've got it.

MR MYERS: My Lord, again, I find myself rising because we've got the evidence of Rebecca Morgan here and this was dealt with her. She was at work until about 7 pm that day and, when asked about these charts, accepted that is her writing at 18.00 and we can see that.

MR JUSTICE GOSS: It's the same writing as the previous entries, which she signed and are countersigned by the defendant.

MR MYERS: Yes.

MR JOHNSON: Absolutely, but the issue is who does the feed and the witness, who has the chart in front of her, is saying, "I did it". Much as Mr Myers keeps jumping in to help the witness, I'd much rather have the witness's answer unless Mr Myers wants to take the oath as well.

MR MYERS: It's not to help, it's to be fair in the presentation of the evidence. This quote is being put to the witness about the evidence that don't reflect what the evidence was.

MR JUSTICE GOSS: There are two different points here: you're making one point, Mr Johnson is making another point about it or other points about it. Anyway, the jury have heard the evidence, they can see the chart for themselves, they can see what the defendant said when she was asked about it with the chart apparently in front of her and they can hear what she says now and it's for them to assess it all.

MR MYERS: Well, I agree with that, my Lord.

MR JUSTICE GOSS: Right. Anything else on this particular topic?

MR JOHNSON: No, thank you.

MR JUSTICE GOSS: Thank you very much.

All right, we'll break off until 2.10 then, please, members of the jury.

(1.04 pm)

(The short adjournment)

(2.15 pm)

(In the absence of the jury)

MR JUSTICE GOSS: Mr Myers.

MR MYERS: We're grateful. We are, as I understand it, ready to continue. Ms Santolini has seen Ms Letby over the lunchtime break.

MR JUSTICE GOSS: Good.

MR MYERS: Ms Letby would like to continue. Ms Santolini raises nothing through me to say that can't happen, subject, as ever, to us keeping watch on how Ms Letby is. Save for that, there's nothing that I add.

Ms Santolini is available if your Lordship wishes to have her brought into court, but I'm relaying the key points your Lordship needs to know.

MR JUSTICE GOSS: Thank you, Mr Myers, no, I don't.

Thank you very much, I'm very grateful. I will, of course, as I have been, keep an eye on Ms Letby and we will have our conventional quarter of an hour break as well this afternoon.

MR MYERS: Thank you.

MR JUSTICE GOSS: All right. I'll have the jury in then, please.

(In the presence of the jury)

MR JUSTICE GOSS: Sorry we're a little late starting.

Yes.

MR JOHNSON: My Lord.

So far as your evidence before the break was concerned, having thought about it over the adjournment, is there anything that you would like to amend or add to or --

LETBY: No.

JOHNSON: We'd got almost to the position, or pretty much to the position, where you were handing over [Baby P] to one of your colleagues at the end of the day shift of 23 June on which [Baby O] had died. All right?

LETBY: Yes.

JOHNSON: You said in answer to questions from your own counsel that so far as [Baby P]'s position at handover at between 19.30 and 20.00 hours on the 23rd, there were no problems.

LETBY: Yes.

JOHNSON: Is that your position?

LETBY: Yes.

JOHNSON: But do you remember the evidence of the X-ray, the radiograph that was taken of [Baby P] just after 8 pm?

LETBY: Yes.

JOHNSON: And do you remember the evidence of Dr Arthurs about this?

LETBY: No.

JOHNSON: Do you remember it being suggested to Dr Arthurs on your behalf that the reason for the abdominal distension was because [Baby P] had been on CPAP?

LETBY: I can't recall, but if that's what he said, then...

JOHNSON: As a matter of fact, the last time [Baby P] had had CPAP was about 36 hours earlier, wasn't it? Do you remember that?

LETBY: Again, I can't say without looking definitively, no.

JOHNSON: Well, would you like to look at the chart to see?

LETBY: No, if it's agreed then that's okay.

JOHNSON: Well, it can be checked. It's J23820. I am suggesting that the last time [Baby P] had had CPAP was 11.45 hours on Wednesday the 22nd, so just before midday the previous day.

I don't think it's in the hard copies, my Lord.

MR JUSTICE GOSS: Right. Well, all right. It can be checked. If someone else wants to check it, but I think we'll proceed. We won't spend time looking at it now.

MR JOHNSON: No.

If we put up tile 173, please, Mr Murphy. We see there that:

"The NG tube is in a satisfactory position, but there are gas-filled loops throughout the abdomen, through to the lower rectum with no evidence of obstruction and no signs of perforation."

And that was because in the aftermath of his brother, [Baby O]'s death, you had pumped air into [Baby P] just before you handed him over, isn't it?

LETBY: No.

JOHNSON: This was a significant change from the position with [Baby P] in the preceding 48 hours though, wasn't it?

LETBY: Yes.

JOHNSON: Yes. And it happened just as you were handing him over, didn't it?

LETBY: That's when the X-ray was taken, yes.

JOHNSON: Yes. It's very similar, isn't it, to what had happened with [Baby O] the previous night? Just as you handed him over [Baby O] was having problems. Do you agree?

LETBY: Yes, [Baby O] had problems on into the night shift, yes.

JOHNSON: So 24 hours later, problems for a child who had had no problems during the day?

LETBY: Yes.

JOHNSON: Yes. And Dr Arthurs, you may remember, told us that this image was quite unusual in a baby of this age and gestation. Do you agree that the gas was there because somebody put it there?

LETBY: I can't comment on how the gas got there. I just know that it wasn't me that put it there.

JOHNSON: Look at T171, please. It's behind a different tile, but the same image that we've looked at before; is that right?

LETBY: Yes.

JOHNSON: We can see that throughout the day when you are signing or countersigning the feeding record, there are no significant aspirates, can't we?

LETBY: Yes, that's right.

JOHNSON: Yet we see the moment, literally the moment he's handed over, a 14ml aspirate?

LETBY: Yes.

JOHNSON: And that was your doing, wasn't it?

LETBY: No.

JOHNSON: And on your way home, you were sowing the seeds with your colleagues? Do you remember that text I showed you at tile 403 on the [Baby O] sequence, "Worried because they're identical", or words to that effect?

LETBY: Yes.

JOHNSON: Yes, just as you got home. Do you remember you were in a two-way conversation, one with [Nurse E], the other with [Dr A]?

LETBY: Yes.

JOHNSON: You were feeding a false narrative to your colleagues, weren't you?

LETBY: No.

JOHNSON: Trying to divert attention away from your homicidal activities?

LETBY: No.

JOHNSON: Do you agree that Dr Mayberry put [Baby P] nil by mouth in the light of those aspirates?

LETBY: Yes. Yes, after midnight, yes.

JOHNSON: After midnight. Do you remember that -- we may have to move on to 23981 -- I'll just find the tile number, I've not written it down, I'm afraid. It's tile 244.

Do you remember being asked about this sequence of events, so far as feeding was concerned, overnight by your counsel as part of your evidence-in-chief?

LETBY: Yes.

JOHNSON: You remember being shown this? This shows, doesn't it, and we heard evidence from her, that Kathryn Percival-Calderbank got an aspirate at 04.00 hours?

LETBY: That's not on this page.

JOHNSON: 23982, sorry. It's tile 244, hopefully. That's it there, isn't it? Can you see?

LETBY: Yes.

JOHNSON: So this is at 4 am.

LETBY: Yes.

JOHNSON: And do you remember saying in answer to your own counsel that the material that was drawn out in the aspirate wasn't there when the earlier aspirate was taken because when the earlier aspirate was taken they would have emptied the stomach because the syringe has a resistance? This was your evidence.

LETBY: Yes.

JOHNSON: Do you remember saying that?

LETBY: Yes.

JOHNSON: But what we can see if we look at it, just examining your explanation, your assertion that this is a developing problem and the reason you've just given for why that is, what we see looking at that chart is that at 7 am, 07.00, do you see that Sophie Ellis got 5ml of air and 2ml of milk at 7 o'clock?

LETBY: Yes.

JOHNSON: How much milk had [Baby P] been fed overnight?

LETBY: I think he'd had two feeds prior to midnight, 28ml was it, or something?

JOHNSON: Yes, but he'd been fed nothing at all, had he, during this day?

LETBY: No.

JOHNSON: So unless somebody was surreptitiously feeding him milk when you weren't there, the obtaining of that milk shows that the stomach hadn't been emptied by Kathryn Percival-Calderbank, doesn't it?

LETBY: No, what I was suggesting is that air would come out when you're aspirating milk. Milk obviously sits in the stomach and

would depend how close it was to the end of the NG tube, but air should come out regardless.

JOHNSON: Well, it all depends where the end of the NG tube is, doesn't it? Because if the --

LETBY: If the NG tube is in the stomach air will come out.

JOHNSON: No, if the NG tube is in the liquid in the stomach it wouldn't take any air out at all until the liquid that it's actually dipped into has been emptied, would it?

LETBY: No.

JOHNSON: It's common sense, you don't need to be a nurse to understand it. It's like emptying a pond with a hosepipe, syphoning a can of petrol. The point I'm trying to make is, so that you understand, is I'm examining your assertion, and the reasons for it, that Kathryn Percival-Calderbank had emptied the stomach with that 25ml aspirate and I'm suggesting that she hadn't.

LETBY: Okay.

JOHNSON: Well, do you agree or disagree?

LETBY: I disagree.

JOHNSON: Do you agree that by the end of this shift on which [Baby P] had problems, he was comfortable, settled and seemed like a well baby?

LETBY: At 8 o'clock in the morning?

JOHNSON: Yes.

LETBY: Um, I'm not sure.

JOHNSON: You're not sure.

LETBY: He had just been reviewed by the registrar, I'm not sure.

JOHNSON: Do you agree that whatever had been the problem when you handed over [Baby P], it had resolved itself by the time you got him back at 7.30 the following morning?

LETBY: No, because he's been placed nil by mouth.

JOHNSON: Yes, but he was a well baby by the time he was handed over to you, wasn't he?

LETBY: No, I disagree. A baby nil by mouth is not a well baby.

JOHNSON: Okay. Do you remember saying in evidence that Sophie Ellis was quite concerned about [Baby P] in view of what had happened to [Baby O] and that he'd been reviewed several times overnight?

LETBY: Yes.

JOHNSON: Do you remember what you said to the police though?

LETBY: No.

JOHNSON: Well, can we look at your interviews, please. It's the [Baby P] interviews, [Baby P]. It's the first interview. So [Initials of Baby P] 2, please this is an issue on 5 July 2018. Can you see the second question on the page says:

"Okay, but in terms of your view of his health on this day, on the 24th, when you first took over his care, was he stable, was he well?"

And your answer, please?

LETBY: "He appeared so, yes. I think it was just we were keeping an eye -- he was nil by mouth at the time and keeping an eye on his abdomen".

JOHNSON: I have just made that suggestion to you that he was stable and he was well and you said that he wasn't. Why did you say that he was in the interview?

LETBY: I didn't say he was stable, I said that he wasn't a well baby, being nil by mouth.

JOHNSON: "Was he stable? Was he well?" --

LETBY: Yes, this is the time of interview, yes.

JOHNSON: "He appeared so, yes."

LETBY: Yes, I agree. This was fresh in my memory, more so than today.

JOHNSON: Are you trying to make the position of [Baby P] appear to be worse now to explain what happened whilst you were looking after him on 24 June?

LETBY: No.

JOHNSON: Let's go to -- put the interviews away for now, please, and go to the sequence of events at tile 258, please.

There we see the population distribution, don't we?

LETBY: Yes.

JOHNSON: I just want, if we can, to go back to the previous tile, which is 257. What I would like you to do is look at the staff who were on, the registered nurses, and tell us from whom the shift leader would have been selected.

LETBY: The shift leader for this shift had been selected by the shift leader of the night shift previous to this.

JOHNSON: I understand, but which of those names would have been sufficiently qualified to have been a shift leader?

LETBY: [Nurse D] and Chris Booth.

JOHNSON: So it's got to be either one or the other?

LETBY: Yes.

JOHNSON: All right. Can we go back to 258, please. Now, looking at the population layout, we have [Nurse D] with two in nursery 1 --

LETBY: Yes.

JOHNSON: -- together with your friend Mary Griffith. Chris Booth with a single baby in nursery 2 and you with [Baby P].

LETBY: Yes.

JOHNSON: Then Mary Griffith and Angela McShane sharing the babies in nurseries 3 and 4?

LETBY: Yes.

JOHNSON: Does it look likely therefore that the shift leader on this shift was Christopher Booth?

LETBY: Yes, it was Chris Booth, yes.

JOHNSON: Yes.

MR JUSTICE GOSS: You can remember that?

LETBY: Yes.

MR JOHNSON: Now, whilst you were conducting safety checks at the beginning of the shift, you told the jury, in answer to questions from your own counsel, that [Baby P]'s abdomen was quite loopy, that you could see visible loops of bowel, that you spoke to the nurse in charge, who we now know would have been Christopher Booth.

LETBY: Yes.

JOHNSON: And:

"We [you said] were going to wait for Dr Ukoh to review him."

LETBY: Yes.

JOHNSON: Do you remember saying that? Dr Ukoh did that and was still in the nursery when [Baby P] had an apnoea and you were there at the time.

LETBY: Yes.

JOHNSON: And you said you thought Rebecca Morgan was there, too.

LETBY: Yes.

JOHNSON: So that was, in broad terms, your explanation for the first event. But before we get to the detail I just want to ask you the questions I have asked you in relation to all these incidents.

Are you suggesting, looking at the population distribution, that staffing levels caused or contributed to [Baby P]'s collapse and death on 24 June?

LETBY: No.

JOHNSON: Are you suggesting that anybody's mistakes caused or contributed to [Baby P]'s collapse and death?

LETBY: No.

JOHNSON: Is there anything you can point to about the circumstances of [Baby P]'s care that, in your view, caused or contributed to his collapse or death?

LETBY: I think from the evidence there was potentially some issues with the chest drain, but I can't say what effect that had on [Baby P], if any.

JOHNSON: Right. Do you accept that when you took over [Baby P], the NGT, the nasogastric tube, was on free drainage?

LETBY: I can't say from my memory. That would be standard practice.

JOHNSON: Yes. Well, let's look at the document. I was just wondering if you could remember. It's 249, please.

This is Sophie Ellis' note. So this is -- it's about 50 minutes before you came into work, isn't it?

LETBY: Yes.

JOHNSON: "Abdomen has been soft and non-distended. 25ml of air aspirated by Senior Nursing Practitioner Kate Ward."

Which is what we saw on the form; is that right?

LETBY: Yes.

JOHNSON: Then:

"NGT placed on free drainage."

LETBY: Yes.

JOHNSON: And that would have remained the position, all things being equal, wouldn't it?

LETBY: Yes.

JOHNSON: Do you accept the evidence of Drs [Dr B] and Mayberry that they discussed [Baby P] at the handover this morning and there were no concerns about him?

LETBY: Yes, that's their discussion, yes.

JOHNSON: Why was it then that you created a false nursing note, which I will show you, at tile 263?

"NG tube on free drainage."

Do you see that at the bottom?

LETBY: Yes.

JOHNSON: "Trace amount in tube. Abdomen full, loops visible."

LETBY: Yes.

JOHNSON: That is not the picture in the note of Sophie Ellis at 6.39, is it?

LETBY: No.

JOHNSON: No:

"Abdomen full, loops visible."

This is a carbon copy, isn't it, of Sophie Ellis' note from the previous day for [Baby O]?

LETBY: No.

JOHNSON: Let's go back to [Baby O]'s sequence if we can, tile 105:

"Abdo looks full. Slightly loopy. Abdo soft."

What you wrote the next day for his brother:

"Abdomen full, loops visible. Soft to touch."

LETBY: Yes. That's what I saw.

JOHNSON: It wasn't what Sophie Ellis had seen 50 minutes earlier?

LETBY: No.

JOHNSON: No. This was a danger sign, wasn't it, if it was true?

LETBY: Yes.

JOHNSON: So what did you do about it?

LETBY: I informed the nurse in charge and the ward round was about to begin, so we waited for the ward round.

JOHNSON: So you waited an hour or an hour and a half for a doctor to attend?

LETBY: Yes.

JOHNSON: You are lying, aren't you?

LETBY: No.

JOHNSON: If you go to the interview, please, it's [Initials of Baby P] 6, so the first page of the [Baby P] interviews. It's bundle 2. Here you were being asked questions about this, weren't you. Can you see?

LETBY: Yes.

JOHNSON: The second question, it's the fourth block from the top of the page:

"Right, and what was the time between you making these observations about his full abdomen and the loops and the ward round and the further observations?"

You say:

"I think the abdomen being full was from when I took over care at 8 o'clock."

LETBY: Yes.

JOHNSON: So this would have been apparent to Sophie Ellis as well then, wouldn't it?

LETBY: Yes.

JOHNSON: And yet no note made by her?

LETBY: No.

JOHNSON: If this was true you would have escalated it urgently and immediately, wouldn't you?

LETBY: I did, I spoke to the shift leader.

JOHNSON: Do you remember what you were telling your friend [Dr A] at this time?

LETBY: No.

JOHNSON: Well, I will remind you. Tile 267, please -- let's start with 264, actually, sorry. So this is following on from a message that [Dr A] had sent you just before 8 o'clock, saying that he'd overslept. All right?

LETBY: Yes.

JOHNSON: Do you remember?

LETBY: Yes.

JOHNSON: You responded:

"Eek, not like you to be late! Have you made it in? Yes, I think they know but they want to talk to [Dr B] today."

That's a reference to the triplets' parents, isn't it? Yes?

LETBY: Yes, I think so, yes.

JOHNSON: "I've got [Initial of Baby R] and [Initial of Baby P]."

Who are [Initial of Baby R] and [Initial of Baby P]?

LETBY: Is it [Baby R]?

JOHNSON: Yes. That wasn't actually true, was it?

LETBY: No. Well, I don't know.

JOHNSON: Well, we've just seen the population distribution, haven't we?

LETBY: Yes.

JOHNSON: We saw that [Baby R] was being looked after by Christopher Booth --

LETBY: Yes.

JOHNSON: -- the shift leader. Why were you telling that falsehood to your friend [Dr A]?

LETBY: I can't answer that. I'd have to see if perhaps I was doing anything with [Baby R] in the charts at that time.

I don't know.

JOHNSON: "[Initial of Baby P] has stopped feeding as large asps."

LETBY: Yes.

JOHNSON: "I've got my student again but might see if she can work with someone else as I don't feel I'm in the frame of mind to support her properly and got loads of paperwork to finish off."

Then this at 267 -- well, let's deal with his response, actually, 266, please:

"Not far away. Should be in just before the handover. Large asps? I wonder if they've all been exposed to a bug that benzylpenicillin and gentamicin didn't account for? Are you okay? See you later."

Then you, so this is now 8.19, tile 267, 20 minutes after you have spotted these loopy bowels in a child whose brother had died the previous day.

LETBY: Yes.

JOHNSON: What do you say?

LETBY: "Hmm maybe. I'll be watching them both like a hawk. I'm okay, just don't want to be here really. Hoping I may get the new admissions. Guess what? Wanted me to go to MRI as Mary looking after baby but has metal so can't go. I've escaped though, phew."

JOHNSON: Did your hawk eye miss the loopy bowels?

LETBY: No.

JOHNSON: Why didn't you tell [Dr A] who was about to arrive?

LETBY: No, [Dr A] was not present on the unit on this day.

JOHNSON: Well, he was on his way into the hospital, wasn't he?

LETBY: Yes, the children's ward, yes.

JOHNSON: "What should I do, [Dr A]? I've got a child here who's exhibiting exactly the same symptoms as his brother did yesterday"?

LETBY: No, I disagree. I raised it with the shift leader and Dr Ukoh was about to start the ward round at that time.

JOHNSON: The reason you didn't raise it is because your note is a fabrication, isn't it?

LETBY: No, Dr Ukoh saw loops as well.

JOHNSON: 267 we've just looked at. Do you remember telling him that you wanted the new admissions?

LETBY: Hoping, yes.

JOHNSON: Yes. 267:

"Don't want to be here really. Hoping I may get the new admissions."

What does that mean?

LETBY: I didn't really want to be in work on that day after what had happened with [Baby O] and I was hoping that maybe I would go into a different nursery where I wasn't with the triplets.

JOHNSON: You say Dr Ukoh saw a loopy bowel; yes?

LETBY: I think -- well, I can't recall his exact note.

JOHNSON: I'm sorry, you just (inaudible) what you said. I said:

"The reason you didn't raise it is because your note is a fabrication."

You replied:

"No, Dr Ukoh saw loops as well."

LETBY: That's my belief, yes.

JOHNSON: Well, let's see what the evidence is. Tile 289, please:

"On examination: mildly pale, no recession, heart sounds normal plus very soft systolic. Abdomen moderately distended/bloated, soft."

No mention of loopy bowel there, is there?

LETBY: No.

JOHNSON: If we scroll down to remind ourselves, please, further down. Why didn't you tell him about the loopy bowel?

LETBY: It's something that -- it would have been visible at that time.

JOHNSON: Well, it wasn't, was it, because he didn't see it?

LETBY: Or he's just not recorded it.

JOHNSON: Or you have misrecorded it.

LETBY: No.

JOHNSON: No real issue there, is there, in his medical note?

LETBY: He's slightly mottled, he's got a slightly distended abdomen, he's having some bradys and desats.

JOHNSON: Do you remember that as soon as he moved on to a different child, Dr Ukoh said that [Baby P] collapsed?

LETBY: Yes.

JOHNSON: So not only has the night nurse resolved the problem, which emerges as soon as you take over, but the doctor who examined the child doesn't see what you say you saw, doesn't identify a significant problem, and yet within 5 minutes the child has collapsed?

LETBY: Yes.

JOHNSON: And that has to be your doing, doesn't it?

LETBY: No.

JOHNSON: Do you remember Rebecca Morgan suggesting that you had left the room at this point?

LETBY: Yes.

JOHNSON: Was that true?

LETBY: I can't comment on Rebecca's statement. From my memory, I was in the room. That's all I can comment on.

JOHNSON: Were you trying to get out of the way so you weren't immediately associated with yet another collapse on your watch?

LETBY: No, I think I've always been quite clear that I thought I was in the nursery at the time.

JOHNSON: Why would you have left the nursery?

LETBY: There could be many reasons why I would have left the nursery.

JOHNSON: Well, there could be potentially, but you didn't have any other children to look after, did you?

LETBY: No.

JOHNSON: Were you worried that [Baby P] was high risk?

LETBY: When I took him over for the care that day?

JOHNSON: Yes.

LETBY: Yes.

JOHNSON: But you left him?

LETBY: No, I don't agree. I've not said that I've left him.

JOHNSON: If we look at your nursing note at tile 286, please.

Can we go to the original, please? What you record there is what happened from 8 am. He was in an incubator, nil by mouth. You speak of the fluids, the IV fluids that were running at a certain rate. The line was occluding. The NG tube was on free drainage:

"Trace amount in the tube. Abdomen full, loops visible."

Which we've already dealt with. Then this:

"Registrar Ukoh arrived to carry out ward round. [Baby P] had an apnoea/brady/desat with mottled appearance requiring facial oxygen and Neopuff for approximately 1 minute."

That's a rather different picture to what's recorded in Dr Ukoh's note, isn't it?

LETBY: I'm not sure without looking at his note.

JOHNSON: Okay, well, we'll go back to it. 289, please. Can we go to the original? Dr Ukoh's got pretty good writing.

It's further down, please.

He starts with the background; is that right?

LETBY: Yes, this is the note that he's made on the ward round.

The apnoea happened afterwards.

JOHNSON: I know, but the -- that's exactly my point. The impression you are creating in the nursing note is it's as Dr Ukoh arrives that all these events happened.

LETBY: Yes, he was in the room at the time, yes.

JOHNSON: No, no, but you have missed out in your narrative the fact that there was a perfectly normal ward round and it was only after the doctor had moved on that the child collapsed.

LETBY: I don't see what the difference is. He was in the room.

JOHNSON: Very well. Just going back to your note, please -- I'm sorry to jump around -- 286. That note gives the impression, doesn't it, that it was the Neopuffing that caused the abdomen to become distended?

LETBY: That it was becoming increasingly distended, yes.

JOHNSON: Yes. But it's deliberately written, I am suggesting, to create the impression that the distension is the result of the Neopuffing.

LETBY: Okay.

JOHNSON: Do you agree?

LETBY: Yes.

JOHNSON: Yes.

LETBY: It's becoming more distended, yes.

JOHNSON: There was no mention of Neopuffing to Dr Ukoh when he arrived, was there?

LETBY: Dr Ukoh was present.

JOHNSON: Do you remember Dr Ukoh saying that in that 5 minutes between the ward round examination and the collapse, [Baby P] was in a very different condition?

LETBY: No. Different? No, I don't understand.

JOHNSON: Yes, the baby he saw at 9.35 was in a very different condition to the baby he saw at 9.40.

LETBY: Yes.

JOHNSON: And the only conceivable reason for that, I'm suggesting, is because you sabotaged him.

LETBY: No.

JOHNSON: He also said that you were very keen for [Dr A] to be called.

LETBY: Yes, in his opinion, yes.

JOHNSON: Yes. And you've just told us that [Dr A] wasn't even due to be on the neonatal unit.

LETBY: No, he wasn't.

JOHNSON: No.

LETBY: [Dr B] requested for me to call him.

JOHNSON: Oh, it was [Dr B] who wanted [Dr A], was it?

LETBY: Yes, it was discussed that he had been present with [Baby O] the day before and that [Dr A]'s expertise was needed.

JOHNSON: That wasn't quite the way Dr Ukoh put it, was it?

LETBY: No. That's Dr Ukoh's opinion.

JOHNSON: Yes. Well, he was there, of course, wasn't he?

LETBY: Yes.

JOHNSON: Were you trying to attract [Dr A]'s attention?

LETBY: No.

JOHNSON: So it was [Dr B], let me get this right, who discussed with who about getting [Dr A]?

LETBY: I believe it was myself and Chris and Dr Ukoh and she had raised that [Dr A] had been present the day before for [Baby O], so it might be worthwhile that he came as a senior registrar to assist.

JOHNSON: Oh right. The defence statement at paragraph 182 says this:

"A little after 9.35, I was talking with Rebecca Morgan when [Baby P] stopped breathing. Dr Ukoh was in nursery 2 already because he was writing up his notes. The monitor sounded and I shouted for [Dr B] because she was already on the unit. I also called [Dr A] to assist."

LETBY: Yes.

JOHNSON: No mention there of getting [Dr A] being the idea of [Dr B].

LETBY: No.

JOHNSON: Why not?

LETBY: This statement doesn't cover every single point that I make.

JOHNSON: No. Did you enjoy being in these crisis situations with [Dr A]?

LETBY: No.

JOHNSON: Did it give you something to talk to him about and message him about?

LETBY: No. [Dr A] and I were friends.

JOHNSON: Something in common that you could share?

LETBY: No.

JOHNSON: [Baby P] underwent full CPR with four doses of adrenaline.

LETBY: Yes.

JOHNSON: But he was brought back, wasn't he?

LETBY: Yes.

JOHNSON: And the reason he crashed was because you had injected air down the nasogastric tube.

LETBY: No.

JOHNSON: Do you agree with [Dr B] that this cannot have been an infection because [Baby P] recovered so quickly?

LETBY: Yes, if that's her medical opinion, yes.

JOHNSON: Let's move on to the second episode, please, which happened at 11.30 that morning. This was an occasion when [Baby P] had further CPR. He was put on to a ventilator but he, within a short period of time, was fighting the ventilator. Do you remember [Dr B] saying he was very vigorous and breathing against the ventilator?

LETBY: Yes.

JOHNSON: Is that an accurate summary of what happened?

LETBY: I recall that he needed pancuronium to settle him, yes.

JOHNSON: Yes. And the pancuronium is at tiles 391 and 392, if we can go to that, please. Recognise the writing?

LETBY: Yes.

JOHNSON: Whose is it?

LETBY: It's mine.

JOHNSON: And where was this found?

LETBY: In my house.

JOHNSON: Whereabouts in your house?

LETBY: Was it in the bag? I don't know.

JOHNSON: How did it get there?

LETBY: I put it there.

JOHNSON: Why did you put it there?

LETBY: I collect paper and that's where it's ended up. I put it there.

JOHNSON: Why do you collect paper?

LETBY: I have difficulty with throwing anything away, particularly paper.

JOHNSON: Is there something comforting about paper?

LETBY: No, I keep -- well, I keep paper, yes, from a variety of different sources, yes.

JOHNSON: But not bank statements?

LETBY: No.

JOHNSON: 11.32, pancuronium. How quickly does pancuronium take effect?

LETBY: Um... I'm not sure entirely. That's something I have not used very often, so I'd have to ask a medical opinion for that.

JOHNSON: It's very quick, isn't it, a minute or two?

LETBY: I can't say that. I don't know.

JOHNSON: The purpose of it is to, in effect, paralyse the patient, isn't it?

LETBY: Yes.

JOHNSON: It enables the machine to take over the breathing function?

LETBY: Yes.

JOHNSON: And it prevents the patient expending wasted energy by fighting the ventilator?

LETBY: That's right.

JOHNSON: Shortly afterwards, at about noon, after [Baby P] had been moved into nursery 1, you --

LETBY: He didn't move into nursery 1.

JOHNSON: Sorry?

LETBY: He didn't move into nursery 1.

JOHNSON: Didn't move into nursery 1? Right. Well, wherever he was, you were with [Dr B] and [Baby P] and [Dr B] told you that he was -- that the transport team was on the way.

LETBY: Yes.

JOHNSON: Yes? Do you remember what you said?

LETBY: I know what she says that I said. I don't recall saying that.

JOHNSON: "He's not leaving here alive, is he?"

That, as a matter of fact, was an accurate prediction, wasn't it?

LETBY: I don't recall saying that statement at that time.

JOHNSON: What did you say?

LETBY: I don't recall the conversation.

JOHNSON: Do you remember [Dr B] told the jury that [Baby P] had just had a good blood gas?

LETBY: Yes.

JOHNSON: Was [Dr B] correct when she said that people in the medical world just don't say things like, "He's not leaving here alive, is he"?

LETBY: Yes.

JOHNSON: It's just not the done thing?

LETBY: No.

JOHNSON: Do you remember that there was a further desaturation at 12.28?

LETBY: Yes.

JOHNSON: The third --

LETBY: Yes.

JOHNSON: -- and what I called the tea room desaturation. Do you remember that?

LETBY: Yes.

JOHNSON: This is when [Dr B] and, I think it was, was it [Dr A], had gone to the tea room --

LETBY: Yes.

JOHNSON: -- for a quick break? It was at 12.28 and you were still looking after [Baby P]?

LETBY: Yes, I was [Baby P]'s nurse throughout the day, yes.

JOHNSON: Yes. [Baby P] had been given pancuronium an hour earlier --

LETBY: Yes.

JOHNSON: -- hadn't he? So he was fully immobilised?

LETBY: Yes.

JOHNSON: And the next thing that Drs [Dr A] and [Dr B] heard was you shouting for help?

LETBY: Yes.

JOHNSON: You, when they went into the nursery, were standing over [Baby P]? Weren't you?

LETBY: I can't recall that now, but that's what they say.

JOHNSON: And there was a problem with his breathing tube; do you remember that?

LETBY: I don't remember it from my memory now, no.

JOHNSON: This was a child that could not move, so he could not have dislodged his own tube, could he?

LETBY: No.

JOHNSON: This was a tube that had gone in at 9.50?

LETBY: Yes.

JOHNSON: So about 2.5 hours earlier?

LETBY: Yes.

JOHNSON: The problem, in inverted commas, with the tube happened just after the doctors had left the room, didn't it?

LETBY: Yes.

JOHNSON: Just after you had said, "He's not getting out of here alive"?

LETBY: I don't agree that I said that.

JOHNSON: Whether or not you said that, is this yet another bit of bad luck that it happened just after everybody else left?

LETBY: Yes.

JOHNSON: And from this moment on, events have demonstrated that [Baby P] was in a terminal decline; would you agree?

LETBY: Yes.

JOHNSON: The damage had been done, whatever it was. Do you accept Dr Brearey eliminated a heart issue?

LETBY: Yes.

JOHNSON: You will remember, I'm sure, the final terminal collapse of [Baby P] at 15.14?

LETBY: Yes.

JOHNSON: And you will remember, I'm sure, that his father, [Father of Babies O, P & R], said that the circumstances of [Baby P]'s death were very similar to [Baby O]'s?

LETBY: Yes.

JOHNSON: Just as I am suggesting you had predicted, both in your text on the way home after [Baby O]'s death and in your remark to [Dr B].

LETBY: No, that's not what happened.

JOHNSON: Well, certainly it's what was in the text, isn't it?

LETBY: Yes.

JOHNSON: That was a portent of doom, wasn't it?

LETBY: No, I disagree.

JOHNSON: You disagree. Did you enjoy making predictions when you knew what was going to happen?

LETBY: No.

JOHNSON: And you got very excited in the aftermath of [Baby P]'s death, didn't you?

LETBY: No, I did not.

JOHNSON: Two babies killed within about 24 hours of each other.

LETBY: No.

JOHNSON: And you were acting, according to [Dr B], in a totally inappropriate way in that room?

LETBY: According to [Dr B], yes.

JOHNSON: Just as you had acted in an inappropriate way with [Mother of Baby I] and with [Mother of Baby C]; do you agree?

LETBY: No, I don't.

JOHNSON: And you were falling over yourself to tell Sophie Ellis about it, weren't you?

LETBY: No, I told Sophie as respect (sic) for her coming in on that shift.

JOHNSON: Out of respect?

LETBY: It's common practice that we would let staff know so they don't just walk in and find something out or potentially bump into a parent and say something.

Usually, we would tell a member of staff.

JOHNSON: Oh, right. So she was about to come into work, was she?

LETBY: I can't remember if she was coming into work or if she was off, I can't remember.

JOHNSON: I think you remember exactly where she was, don't you?

LETBY: No.

JOHNSON: Well, let's remind you. 629, please -- sorry, 628, which is her texting you.

LETBY: Yes, so I was giving her an update in reference to what she's asked me.

JOHNSON: And she was at the races.

LETBY: Yes.

JOHNSON: Why didn't you just leave her alone?

LETBY: She's asking me.

JOHNSON: Oh yes. Did you enjoy the drama?

LETBY: No.

JOHNSON: 654, please. This is to Sophie later on.

LETBY: Yes.

JOHNSON: "Just blew tummy up."

Where have we heard that before?

LETBY: With other babies.

JOHNSON: Yes, but which one in particular?

LETBY: [Baby O].

JOHNSON: Yes. In his sequence at tile 334, virtually the same message.

LETBY: Yes.

JOHNSON: You'd killed them both, hadn't you?

LETBY: No.

JOHNSON: You were drawing the comparison to your friend [Nurse E] at 654, weren't you?

LETBY: Sorry, this is...

MR JUSTICE GOSS: You mean Sophie Ellis?

MR JOHNSON: I beg your pardon, yes.

LETBY: That's what happened: his tummy blew up and he had apnoeas.

JOHNSON: Similar to [Baby O]?

LETBY: Yes.

JOHNSON: "Worry about them, they're identical."

LETBY: Yes.

JOHNSON: Your portent of doom had fulfilled itself, hadn't it?

LETBY: No.

JOHNSON: At your hand?

LETBY: No.

JOHNSON: Do you remember that [Baby P] also had a liver injury, albeit it wasn't as bad as [Baby O]'s?

LETBY: Yes.

JOHNSON: You caused that, didn't you?

LETBY: No.

JOHNSON: And you injected him with air down the NGT?

LETBY: No, I didn't.

MR JOHNSON: Well, my Lord, that may be a good moment because we're about to move on.

MR JUSTICE GOSS: Yes. Can we say 10 minutes? Will that be sufficient?

MR JOHNSON: It certainly is for me.

MR JUSTICE GOSS: Because we've had slightly truncated sessions thus far, so a 10-minute break then, please.

(In the absence of the jury)

MR JUSTICE GOSS: Is there a problem with 10 minutes? I was looking at the escorting member of staff.

DOCK OFFICER: Lucy just said something but I didn't quite hear what she said.

LETBY: It's just by the time we get out and cuffed, I have to come back in.

MR JUSTICE GOSS: You haven't got time to have a comfort break?

LETBY: With 10 minutes, no, because we have to be in 5 minutes before everyone comes back.

MR JUSTICE GOSS: All right. We'll have 15 -- well, we'll have 13 minutes from now. I'll rise at this stage.

MR MYERS: I should have got to my feet and asked for 15 minutes, in fact, my Lord, because that is the time we decided. Even if the hearings are truncated, the purpose of that is to assist Ms Letby and that is not going to change however long the hearing lengths are.

MR JUSTICE GOSS: So be it. We will have 15 minutes.

MR MYERS: I should have said.

MR JUSTICE GOSS: I did see there was a concern but I didn't want to ventilate it publicly. I'll rise now then.

(3.12 pm)

(A short break)

(3.24 pm)

MR JOHNSON: Just before I make a correction or two, is there anything arising out of what you have said today at any stage that you would like to amend or correct?

LETBY: No.

JOHNSON: Well, there are two things that I need to correct. The first is in relation to a text message that you sent to [Dr A] saying that you had [Initial of Baby P] and [Initial of Baby R], so [Baby P] and [Baby R].

We have found a handover sheet that suggests that at the beginning of the shift, despite what it says on the population distribution, you did have them both at the beginning and then Christopher Booth took over [Baby R].

Okay?

LETBY: Okay, thank you.

JOHNSON: Not at all. I need to get it right. The second correction is I think I mistakenly referred to you sabotaging [Baby O] before you handed him over and what I meant to say was [Baby N], not [Baby O]. Okay?

I don't know if you remember that, but I did at some stage suggest that you -- when you had him on a day shift, you handed him over with a problem -- you created a problem just before you handed him over and so on.

MR JUSTICE GOSS: And that was [Baby N] and not [Baby O]?

MR JOHNSON: Yes. I'm not very good with names today for some reason, so I apologise, it's my fault.

Right. [Baby Q], please. If we go back to your defence statement at paragraph 191. The final count, of course, count 22. You begin by saying that you did nothing to harm [Baby Q]. 191:

"I didn't work nights from 22 June and was not on the ward after the 25th so I cannot be the person [Mother of Baby Q] describes at these times."

192:

"On the day of 25 June I was [Baby Q]'s designated nurse. He was in nursery 2. I also had a baby in nursery 1. We were very busy."

193:

"I don't recall the handover. So much had gone on the day before that I don't recall the time between the handover and when [Baby Q] started to deteriorate."

194:

"It appears that [Baby Q] had been receiving feeds regularly before my shift, although aspirates continued to be withdrawn. I don't understand why feeding continued if he was not digesting the milk he was being fed."

195:

"[Baby Q] was due a feed at 09.00 but I did not feed [Baby Q]. I wanted the doctors to see him on the ward round before he was fed. After I had undertaken checks and observations on [Baby Q], I told Mary Griffith that I was going to nursery 1 to look after

the baby there. That baby was BM. Minna Lappalainen was at the desk."

196:

"I understand that whilst I was looking after B [that's presumably BM], [Baby Q] was sick and that he was then Neopuffed by Minna. This was already happening when I came back to the nursery. I conducted some checks and I have noted that I aspirated 'air +++' from his stomach. I don't know how that air came to be there. I didn't do that."

Then 197:

"The doctors reviewed [Baby Q] and put him in nursery 1 where I continued to look after him. I wouldn't say that this was a collapse. [Baby Q] was sick and he did need the Neopuff and suction but he was not ventilated until much later in the day when he became tired."

199:

"I don't remember aspirating [Baby Q] with an NG tube after the vomit. I continued to look after [Baby Q] during the day."

201:

"[Baby Q] didn't have another collapse or become seriously unwell but he did become tired during the course of the day. This is why he was intubated. He was not put on a ventilator because of concerns about NEC."

Finally:

"I did not deliberately retain a handover sheet from the morning of 25 June 2016."

All right?

LETBY: Yes.

JOHNSON: So first of all, is that what the document says?

LETBY: Yes.

JOHNSON: And secondly, do you continue to say that that is the truth?

LETBY: No, there is something I would like to amend and it's just -- it's 191. I think I may have been on shift after 25 June. I'd like to double-check that, please.

JOHNSON: Yes, well, I don't think it's likely to be something that we're going to fall out about.

Now, I don't know whether you remember, but do you remember that Dr Evans was challenged for suggesting that [Baby Q] had crashed? It was suggested to him that, "That is a gross exaggeration of what we're dealing with".

LETBY: Yes.

JOHNSON: Is that your position?

LETBY: Yes.

JOHNSON: Do you accept that crashed is the right word?

LETBY: No, I don't think this was a crash collapse, no.

JOHNSON: What word would you use?

LETBY: It's a deterioration.

JOHNSON: A deterioration?

LETBY: Yes.

JOHNSON: What sort of a deterioration?

LETBY: An acute deterioration at that time.

JOHNSON: Do you remember the text message you sent to [Dr A] about 2 weeks later --

LETBY: No.

JOHNSON: -- on 6 July? Do you remember the one I'm talking about?

LETBY: No.

JOHNSON: Well, we may come to it tomorrow, but it's in the sequence of post-indictment text messages. Do you remember that document?

LETBY: Yes.

JOHNSON: And it's two messages at 163 and 164, if Mr Murphy could help me with that, please.

(Pause)

So this was [Dr A] to you:

"[Initial of Baby Q]'s [in this context, as I think you have already told us, this is [Baby Q]] care is going to be reviewed on Thursday as well because of acute deterioration."

LETBY: Yes.

JOHNSON: And you responded, the next message:

"That makes sense."

This wasn't just some common or garden desaturation, was it?

LETBY: Yes, it was. I'm referring to the decline that came later on in the shift.

JOHNSON: You've just told us that you would characterise what happened at the beginning of the shift as an acute deterioration.

LETBY: Yes.

JOHNSON: And that's what is being talked about here, isn't it?

LETBY: Yes. I would also class what happened to him later in the shift as an acute deterioration as well.

JOHNSON: What's the difference between a crash and an acute deterioration?

LETBY: I would define a crash as something that requires a crash call, an emergency bleep, which means the baby is needing resuscitative measures.

JOHNSON: After [Baby Q]'s acute deterioration, you were relieved of the care of the baby BM, weren't you?

LETBY: Yes.

JOHNSON: Why was that if what happened to [Baby Q] at the beginning of the shift really wasn't anything very serious?

LETBY: Because he needed to move into nursery 1.

JOHNSON: Why did he need to move into nursery 1?

LETBY: Because he deteriorated.

JOHNSON: He was in a serious condition after that crash, wasn't he?

LETBY: No, I disagree, no.

JOHNSON: He needed one-to-one care after that acute deterioration, didn't he?

LETBY: Yes, because he was classed as ITU, yes.

JOHNSON: All right. So this thing that has been characterised as a gross exaggeration to say it's a crash put him into ITU; can we agree on that?

LETBY: Yes.

JOHNSON: Let's look at the population distribution, please. It's at 87 of the [Baby Q] sequence, please.

This is the beginning of the shift, isn't it?

LETBY: Yes.

JOHNSON: What we see is that [Baby Q] was with you in nursery 2 and YM was with Mary Griffith.

LETBY: Yes.

JOHNSON: You had another child, BM, in nursery 1, and [Nurse B] had two others.

LETBY: Yes.

JOHNSON: One of whom was a baby called JA, wasn't he?

LETBY: Yes.

JOHNSON: Do you remember JA as a matter of interest?

LETBY: Yes.

JOHNSON: Did you ever fill in a Datix form for JA?

LETBY: I'm not sure about that.

JOHNSON: Right. We'll come to that a little later.

In nursery 3 were three children, two of whom were being cared for by Mary Griffith?

LETBY: Yes.

JOHNSON: And in 4, four children, three of whom being cared for by the band 4 Nicola Dennison and your friend Mary also in there as well?

LETBY: Yes.

JOHNSON: And so Mary was split, her attentions were split between three separate rooms?

LETBY: Yes.

JOHNSON: Concentrating on [Baby Q], though, is it your case that staffing levels caused or contributed to [Baby Q]'s collapse?

LETBY: No.

JOHNSON: Is it your case that mistakes contributed to his collapse?

LETBY: No.

JOHNSON: Or that some form of medical or nursing incompetence made a contribution?

LETBY: No.

JOHNSON: We'll see what we can agree. At the beginning, following his birth, do you accept or do you agree that [Baby Q] needed some respiratory support initially?

LETBY: Yes.

JOHNSON: So he was born on 22 June at 04.09 and was admitted to the NNU at 04.35, just to jog your memory. That, of course, was the day after [Babies O, P & R] were born, wasn't it?

LETBY: Yes.

JOHNSON: Whilst you were still away, as a matter of fact.

LETBY: Yes.

JOHNSON: [Baby Q] made good progress, didn't he? Do you remember that?

LETBY: Yes.

JOHNSON: He was off CPAP on day 2 of life, which was 23 June. Do you remember that?

LETBY: I know he was on Optiflow by the time I was caring for him on this day, yes.

JOHNSON: Well, yes. Well, it may be a good idea to have a look, actually. Behind divider 22 in the second jury bundle, so it's the one with the big blue 2 on as far as you're concerned.

There are some numbers in the top right-hand corner.

If you go to 008, please. There's a very easy ready reckoner in the gas chart, isn't there, as to the position of [Baby Q]'s breathing support?

LETBY: Yes.

JOHNSON: So we see a couple of CPAPs at 16.57 on the day he was born, followed by 00.55 on the 23rd. But after that, self-ventilating in air?

LETBY: Yes.

JOHNSON: Back on to CPAP after what I'm suggesting was a collapse at your hands at the beginning of the shift on the 25th; right?

LETBY: Yes.

JOHNSON: So according to that, at least, no breathing support?

LETBY: No.

JOHNSON: It's just before you suggested that there was some form of breathing support.

LETBY: I thought from my memory, but that's why I wanted to check.

JOHNSON: Okay. As long as we know.

Now, can we go to tile 72, please. That gives us the observation chart for which there is actually a hard copy as well in that same bundle, pages 2 and 3.

That is a very encouraging picture, isn't it, until 9 o'clock, when these events unfolded?

LETBY: Yes. Other than some temperature issues, but yes.

JOHNSON: On the night shift of the 23rd into the 24th, do you remember the evidence of Tanya Downes, who said that she got a 2ml bile and blood-flecked aspirate at 21.30?

LETBY: Yes.

JOHNSON: His feeds were stopped, but we know, don't we, that [Baby Q]'s bowels were working well because of the information - - if you carry on in that document, it's at page 3B in the top right-hand corner.

LETBY: Yes, he had his bowels opened, yes.

JOHNSON: So that's the position on the 23rd into the 24th.

On the day shift of the 24th, which was the day that [Baby P] died, do you remember [Nurse D] was looking after him and told us that he was fine, albeit there were a few what she described as false bradys from a loose ECG lead?

LETBY: Yes.

JOHNSON: On the night shift of the 24th into the 25th -- so this is the shift before you took over -- Samantha O'Brien looked after [Baby Q]; do you remember?

LETBY: Yes.

JOHNSON: She was getting aspirates but he was tolerating very small trophic feeds?

LETBY: Yes.

JOHNSON: Half a millilitre every 2 hours?

LETBY: Yes.

JOHNSON: His breathing was good; do you remember that?

LETBY: Yes.

JOHNSON: And [Dr D] -- and this is tile 79, please -- told us that at 06.58 there was a good blood gas. Scroll down.

Thank you very much. It's the first blood gas on the 25th of the 6th. Do you see that?

LETBY: Yes.

JOHNSON: Would you agree that that is a very good blood gas?

LETBY: Yes.

JOHNSON: I think you said on the afternoon of 17 May, in answer to questions you were asked by your counsel, that, to quote you directly, talking about this blood gas:

"I was aware this had not been as good as previously."

What are you referring to there?

LETBY: So there has been a slight increase in the lactate.

JOHNSON: But essentially that's it, is it?

LETBY: Well, his pH isn't as good. It's still a good gas, but it's not as good as the previous one, no.

JOHNSON: Do you agree that [Baby Q] was in good condition when you took him on at about 08.00 on 25 June?

LETBY: No, there were concerns for his abdomen and his feeds.

JOHNSON: Who was concerned?

LETBY: I was concerned.

JOHNSON: Anybody else?

LETBY: I don't recall.

JOHNSON: Well, when you took handover, were concerns being expressed to you by anybody else about him?

LETBY: So when I received handover, yes, I'd been informed that he'd had some large aspirates overnight.

JOHNSON: He was in good condition, wasn't he?

LETBY: I don't think a baby having large aspirates is a baby in good condition, no.

JOHNSON: We've already established by looking at tile 87 that your other charge was BM in nursery 1.

LETBY: Yes.

JOHNSON: And when you were interviewed by the police you could still remember BM's name, couldn't you?

LETBY: Yes.

JOHNSON: Yes. So several years, a couple of years later, you could still remember that particular child's name?

LETBY: Yes.

JOHNSON: Do you remember when you were asked about [Baby Q] by your counsel on 17 May, he said this -- it was just after lunch on 17 May when your counsel asked you about [Baby Q]'s condition when you took him over, you said this:

"I don't recall there being any significant problems with [Baby Q]."

And then you were asked the same question again and directed to the observation charts and you said:

"He was on the cold side and in light of the aspirates on the chart I decided I'd prefer to have him reviewed before feeding him."

LETBY: Yes.

JOHNSON: Okay. We've already established that Mary Griffith had several babies to look after in three different rooms, didn't we?

LETBY: Yes.

JOHNSON: Right. I just want to look at Mary Griffith's movements immediately before [Baby Q] had his, what I will call, collapse. So we need to go to the black big file, please, the very last neonatal review. We're going to look at what you were doing and what Mary Griffith was doing. Okay?

LETBY: Yes.

JOHNSON: At line 18, so it's page 2 of 7, we can see that you were doing an infusion for a child, JA, who was in nursery 1 and you were doing that with [Dr A] and [Nurse B].

LETBY: Yes.

JOHNSON: At 08.00 hours, so it's lines 27 and 28, you were doing observations and the neonatal fluid balance chart for [Baby Q].

LETBY: Yes.

JOHNSON: At 08.30, lines 45 and 46, you went back to nursery 1 to do observations and make an entry on the chart for the child BM in nursery 1?

LETBY: Yes.

JOHNSON: At 08.32, do we see that you were co-signing for medication with Mary Griffith for MB who was in nursery 3?

LETBY: Yes.

JOHNSON: At 09.00, lines 60 and 61, we see that Mary Griffith was doing observations for the child YM --

LETBY: Yes.

JOHNSON: -- who is the other child in nursery 2? What we also see, though, at line 59, is an unsigned entry timed at 09.00 hours for the child BM, who was your child.

LETBY: Yes.

JOHNSON: But also, at line 55, just going up the page, we see that you are recording yourself as doing observations for [Baby Q] --

LETBY: Yes.

JOHNSON: -- at 09.00. When you were doing those observations, were you interrupted by Mary Griffith?

LETBY: I don't recall doing the observations. I don't recall being interrupted, no.

JOHNSON: I say interrupted because if we go to the sequence of events, please -- I'm sorry to jump around -- at tile 99, there's swipe data for Mary Griffith coming into the neonatal unit.

LETBY: Yes.

JOHNSON: I'm going to suggest to you that whilst Mary Griffith was out of the unit, you had pumped [Baby Q] with some clear fluid.

LETBY: That didn't happen, no.

JOHNSON: Of course, giving him milk wouldn't have been an option, would it, because he wasn't getting anything more than 0.5 of a millilitre every couple of hours?

LETBY: That's right.

JOHNSON: And if he vomited a large amount of milk it would have been very obvious that something was wrong, wouldn't it?

LETBY: Yes.

JOHNSON: And that's why you chose a clear fluid on this occasion, isn't it?

LETBY: No.

JOHNSON: Is Mary Griffith right when she says that you were still in nursery 2 when she started to feed YM?

LETBY: I can't recall exactly what Mary was doing, but she was in the room with her baby when I left, yes. I don't know exactly what she was doing.

JOHNSON: Well, you see, she wasn't in the room at 9.04 because, if we look at lines 62 and 63, we can see that she's co-signing for medications for JA with [Nurse B] and JA was one of the babies in nursery 1.

LETBY: Sorry, I must have -- my copy says that I'm signing the -
-

JOHNSON: Sorry, it's you, I beg your pardon. I said I wasn't very good with names today.

At 09.04 you were certainly not in the room because you were signing for medications with [Nurse B] --

LETBY: Yes.

JOHNSON: -- in nursery 1.

If we go to tile 97, please, and just scroll down.

Just concentrating on the 9 o'clock entries, why did you do half a job?

LETBY: I can't explain why I haven't filled in the saturations.

JOHNSON: You were interrupted, weren't you, by Mary Griffith?

LETBY: No, I don't know why I haven't filled that in.

JOHNSON: Do you remember the reason that you gave to the police as to why it was you left the room?

LETBY: To go and check on the baby in nursery 1.

JOHNSON: It wasn't quite that, was it?

LETBY: To care for her.

JOHNSON: To do cares.

LETBY: Yes.

JOHNSON: Because I'm a bit prone to making mistakes today, let's just look at what you actually said and check I've got it right. It's the second interview bundle, [Initials of Baby Q] 15, please. It's the third and final interview in the [Baby Q] case. Here you're being asked about these events.

LETBY: Yes.

JOHNSON: You can see at the top of [Initials of Baby Q] 15, you say that you completed the -- they, the police, put to you that you completed [Baby Q]'s observations and he was due a feed but it was not given. You say that you didn't give him anything. They ask you whether it was air that you administered and you then are asked whether you deliberately left the room to blame the collapse on Minna and Mary.

LETBY: Yes.

JOHNSON: I think I've probably made a mistake in my note as to where you mentioned cares. Do you remember telling the police that you went to do cares?

LETBY: Yes, my memory at that time was that I needed to go in to do cares on that baby, yes.

JOHNSON: Do you remember several of your colleagues, I think, have said that for babies in intensive care minimal handling is preferred?

LETBY: Yes.

JOHNSON: And the child BM was one that you had had a fair bit of contact with in the hour or so immediately preceding [Baby Q]'s collapse?

LETBY: Um... I don't know about that.

JOHNSON: Well, if you go --

LETBY: I carried out observations, yes.

JOHNSON: Yes. If you go back to page 3 of the neonatal review.

LETBY: Yes.

JOHNSON: You see lines 45 and -- I took you through these before, I think.

LETBY: Yes.

JOHNSON: Just to remind you, it can be difficult to keep a tab on all this, at 08.30, BM, observations and an entry on the chart.

LETBY: Yes.

JOHNSON: So you'd had contact with this intensive care child about 30 minutes or 40 minutes before [Baby Q] collapsed --

LETBY: Yes.

JOHNSON: -- and you left the room?

LETBY: Yes.

JOHNSON: You were not leaving the room to do cares for that child, were you?

LETBY: Yes, that was my memory at that time, yes, that I needed to go into nursery 1 to see this baby.

JOHNSON: I couldn't see it for looking. You've probably all spotted it anyway on page 15. It's about a third of the way down. Do you see in the left-hand column it says 15:57:57 20 00:24:29?

LETBY: Yes.

JOHNSON: And it's actually just after -- ironically just after the question I read to you and you said:

"No, the baby in nursery 1 was due cares at that time."

LETBY: Yes.

JOHNSON: And that just isn't true, is it, at 09.10, that baby was not due cares?

LETBY: That's what I recalled at that time, yes.

JOHNSON: That's what you?

LETBY: That's what I recalled, my memory being, at the time of the interview, this was why I had gone into nursery 1.

JOHNSON: I know that's what you said to the police, but as a matter of fact it wasn't true, was it?

LETBY: She didn't need a nappy change, no.

JOHNSON: What cares were you doing then?

LETBY: I can't say for definite now what I was doing, but she was an intensive care baby, so I wouldn't have left her for too long a period of time without checking on her.

JOHNSON: Yes, but we've established, haven't we, that you started [Baby Q]'s observations at 9 o'clock?

LETBY: Yes.

JOHNSON: You didn't complete them, so rather than completing one job, you then go to do another job that you haven't documented and there's no suggestion in the documentation -- in the paper work that you actually needed to do, so just explain that, please.

LETBY: My memory at that time was that I'd gone to the nursery because that baby needed something. On reflection, looking at all of this, possibly it was that I went in to do medications with [Nurse B]. I can't say now. My memory at the time of the police interview was that I'd gone back to see BM because she was an intensive care baby that needed to be looked at.

JOHNSON: You were just -- because you had sabotaged [Baby Q], you were just looking for a reason to get out of the room when Mary appeared back in the unit, weren't you?

LETBY: No.

JOHNSON: Do you remember when you were giving evidence on 17 May, you said that you'd told Mary Griffith and Minna Lappalainen that you were going to nursery 1?

LETBY: Yes.

JOHNSON: Why would you do that, leaving a job half done in nursery 2?

LETBY: I don't believe I have left the job half done. I agree I have not written down the saturation reading, but otherwise the job was done.

JOHNSON: Oh right. So you just -- is this an error?

LETBY: Yes, I think it's a mistake that any nurse could make and does make.

JOHNSON: So it just happens to be that a clerical error is made pretty much at the moment that the child has an acute deterioration?

LETBY: Yes.

JOHNSON: And part of the clerical error is you disappearing from the room just before the event?

LETBY: That's not a clerical error, no.

JOHNSON: Do you remember when you were giving evidence on 17 June, you said:

"I heard something going on outside" --

MR JUSTICE GOSS: I think you mean May.

MR JOHNSON: Sorry, May:

"I heard something going on outside so came out of nursery 1."

LETBY: Yes.

JOHNSON: "I could see Minna and Mary Griffith were with [Baby Q], so I immediately went over to [Baby Q]'s cot side."

LETBY: Yes.

JOHNSON: "I was informed he'd vomited by Minna and it was very mucousy."

Do you remember all that?

LETBY: Yes.

JOHNSON: You told the jury that the content of your nursing note was all information that you received from other people.

LETBY: Yes, I was not present at the time that he vomited.

JOHNSON: But you were the person who aspirated "air ++" from his tube, weren't you?

LETBY: Yes, that's what Minna Lappalainen says, yes. I can't recall whether I did or not.

JOHNSON: Well, would you write it down if you hadn't done it?

LETBY: I haven't written it down.

JOHNSON: I'm sorry, yes, she wrote it down, didn't she. I think you write it down -- there's mention in the nursing note at some stage.

LETBY: I may have done it, I can't recall specifically, but yes, I may have aspirated the tube afterwards, yes.

JOHNSON: Let's go to tile 102, please, Mr Murphy. We can look at the original by all means, but you can see he had vomited clear fluid nasally and from mouth.

LETBY: Yes.

JOHNSON: That's what you saw, wasn't it?

LETBY: No, that's the information that was given to me.

JOHNSON: "Mottled ++. Neopuff and suction applied. Reg [Dr A] attended. Air ++ aspirated from NG tube."

That's what you aspirated, isn't it?

LETBY: I can't say definitively whether I did or not, but air was aspirated from the NG tube by someone, yes.

JOHNSON: How had that got there?

LETBY: The baby was Neopuffed, so I would assume it has come from the Neopuff.

JOHNSON: You would assume that, would you, potentially?

LETBY: Yes, because in the sequence of events it would make sense that respiratory effort would be addressed first, which would be the Neopuff and suction, and you'd then go on to aspirate the tube.

JOHNSON: Right. So you'd leave the liquid in the airway, would you, while you used the Neopuff?

LETBY: That's a joint thing, isn't it, Neopuff and suction applied together.

JOHNSON: Sorry?

LETBY: Neopuff and suction were applied together.

JOHNSON: Yes, but don't you need to clear -- I'm no medical person, but don't you actually need to clear the liquid out of the airway before --

LETBY: Yes, if there was still liquid inside, yes.

JOHNSON: It could be very dangerous using a Neopuff on a small neonate who's got liquid in the airway, you could be pumping liquid into the lungs, couldn't you?

LETBY: Yes, if you were convinced that there was still a volume of liquid in their airway, yes.

JOHNSON: This is a child that had just vomited clear liquid ++ -
-

LETBY: Yes.

JOHNSON: -- wasn't it?

LETBY: Yes.

JOHNSON: So that would all have to be cleared out before --

LETBY: No, I disagree. A vomit means it would have left the mouth. He'd vomited over his bedding, I believe.

JOHNSON: So you don't check to see that --

LETBY: I didn't check, no. Minna Lappalainen checked him.

JOHNSON: Somebody told Registrar [Dr A] that there had been a profuse vomit with a desaturation, didn't they?

LETBY: I don't recall. I think that was in his note, yes.

JOHNSON: Yes. Let's look at 104, please. Actually, it's 103.

If we could go to the note, it might be easier, rather than hopping between. That's Dr Ukoh. Scrolling down, please... So this is [Dr A] at 9.17:

"Called to NNU at 9.17. Desaturation. Had just vomited then desaturated to low 60s. Minor bradycardia. Bagged via Neopuff circuit. Sats returned..."

Is that "[Something] to 100%"?

LETBY: Yes.

JOHNSON: It continues if we carry on, please. There's a history.

If we carry on.

There was a concern, wasn't there, that [Baby Q] may have inhaled some liquid?

LETBY: Yes. So whenever a baby vomits, that's always a concern, yes.

JOHNSON: You can see at the bottom, as we're looking at that page, it says, it may be quite difficult to read:

"May need metronidazole if evidence of aspiration."

LETBY: Yes.

JOHNSON: And that's precisely what I've just suggested to you, isn't it?

LETBY: Yes.

JOHNSON: Now, [Dr A]'s examination of [Baby Q] lasted for 53 minutes.

LETBY: Right.

JOHNSON: 9.17 to 10.10. This was no minor everyday desaturation, was it?

LETBY: It needed intervention, but it was certainly not on a scale of needing collapse and resuscitative measures, no.

JOHNSON: Are you saying this wasn't very serious?

LETBY: Yes.

JOHNSON: But it was serious enough to tie up a registrar for 53 minutes?

LETBY: If that's how long he was, yes.

JOHNSON: 9.17 to 10.10, I'm going to suggest to you.

LETBY: Okay.

JOHNSON: You tried to kill [Baby Q] by pumping him with a clear fluid, didn't you?

LETBY: No.

JOHNSON: And afterwards, as you were looking after him, he progressed to CPAP?

LETBY: Yes.

JOHNSON: And then ended up having to be given an endotracheal tube, didn't he?

LETBY: Yes.

JOHNSON: Do you remember handing him over to Amy Davies?

LETBY: Not in any great detail. I know Amy did take over, yes.

JOHNSON: Let's just run through some of the messages you were sending to [Nurse E] whilst you were looking after [Baby Q], who was intensive care. 149, please:

"[Baby Q] on CPAP."

This is all while you are in work, presumably --

LETBY: Yes.

JOHNSON: -- with your intensive care baby?

150:

"Aspirated."

151:

"Who's looking after him? Mary?"

Said [Nurse E], and you replied:

"Me. Minna has taken B off me."

So the child that was in nursery 1 taken off you so you can look after [Baby Q]?

LETBY: Yes.

JOHNSON: "So just got him. Almost had a tube earlier but gases improving."

[Baby Q] was in a very bad way, wasn't he, after this, what you have characterised as being a fairly trivial event?

LETBY: I didn't say it was trivial. He has deteriorated but I wouldn't class it as an event such as others that we've been looking at that has needed, say, chest compressions or immediate ventilation or resus drugs.

I don't think it's on the same level as those incidents.

JOHNSON: I don't think anybody ever suggested it was on the same level as those incidents, but it was a very serious acute deterioration that put him into intensive care.

LETBY: Over the course of the morning, yes.

JOHNSON: Yes.

MR JUSTICE GOSS: "Almost had a tube earlier."

Is that intubation to go on a ventilator?

LETBY: Yes.

MR JUSTICE GOSS: Thought so. Okay.

MR JOHNSON: By the time you handed him over to Amy Davies, [Baby Q] was in a fairly poor state, wasn't he?

LETBY: Yes, he'd become tired and his work of breathing wasn't great, so he was intubated, yes.

JOHNSON: Yes. Just go to Amy Davies' note at 238, please:

"[Baby Q] just been intubated as I arrived on shift."

She sets out the pressures, the fact he was on morphine:

"Capillary blood gas repeated at 21.45. Seen by [Dr D]. To continue on the ventilator. Unsettled at the beginning of the shift but has settled. Temperature is elevated."

The cot was altered and reduced and he became much better, didn't he?

LETBY: Yes.

JOHNSON: So a child that was handed to you, I would suggest in good condition, left your hands on a ventilator in intensive care, but, by the time this note was written at 22.29, was returning to normality?

LETBY: I don't think you can say a baby on a ventilator is returning to normality. He was having good gases and he became more settled, yes.

JOHNSON: Okay. Well, let's look at tile 308. This is 12 hours after you've handed him over at the beginning of the day shift. It's a note made by Amy Davies at the end of her shift.

LETBY: Yes.

JOHNSON: "From 02.00 to 06.00 [Baby Q] was very settled."

LETBY: Yes.

JOHNSON: "Minimal handling carried out. HR reduced.

Temperature."

"Much improved", it says, further down.

LETBY: Mm. He's had a poor gas at 06.23.

JOHNSON: Yes. You'd nearly killed him, hadn't you?

LETBY: No, I hadn't killed him, no.

JOHNSON: Do you remember being aware during the shift that Dr Gibbs was taking an interest in who had been there when [Baby Q] had collapsed?

LETBY: Yes.

JOHNSON: He said that he knew that by asking one nurse, the word would get around.

LETBY: Yes.

JOHNSON: And it did, didn't it?

LETBY: Yes.

JOHNSON: To you?

LETBY: Yes, I walked in on the conversation, yes.

JOHNSON: And you were worried about it, weren't you?

LETBY: Yes.

JOHNSON: Because the net was closing in on you, wasn't it, Lucy Letby?

LETBY: No, I was concerned that I was being held responsible for something that did not happen.

JOHNSON: What were you thinking you were being held responsible for that didn't happen?

LETBY: That I'd been neglectful in leaving the nursery and leaving [Baby Q] unattended, which was not the case, and I was keen to make the point that I had not left him unattended at any point.

JOHNSON: The truth, though, of course is that you had pumped him with liquid and with air, hadn't you?

LETBY: No.

JOHNSON: Because you were trying to kill him?

LETBY: No, that's not right.

MR JOHNSON: My Lord, I'm going to move on to some of the rather more peripheral material, which I hope I'll deal with fairly quickly but probably not today.

MR JUSTICE GOSS: All right. So we'll break off there then and continue at 10.30 tomorrow morning, please. You've heard Mr Johnson say he will complete his cross-examination fairly quickly tomorrow morning.

(In the absence of the jury)

Housekeeping

MR JUSTICE GOSS: I think the door is shut.

MR MYERS: It looks shut from here, my Lord, yes.

I won't descend to the detail, my Lord, but there's an item that we received this morning.

MR JUSTICE GOSS: Of which I'm aware.

MR MYERS: Yes, the admissibility of which is in dispute.

MR JUSTICE GOSS: Yes.

MR MYERS: And it occurs to us that, depending when we get to that, that may be something to deal with even before we start with the jury tomorrow if it's going to be relied upon, and if it relates to that part of the cross-examination, otherwise from the sound of it they'll come in and maybe in a very short compass of time they may go out. I only raise it so we can use the time as effectively as possible if that assists the court. I don't know, it's up to my learned friend, but I don't know if that's something to deal with first thing in the morning before the jury come or they come in a little later or how we deal with it.

MR JOHNSON: I'm more than happy to deal with it at your Lordship's convenience.

MR JUSTICE GOSS: I think it would make sense, it would be a convenient time to deal with it. 10 o'clock?

MR JOHNSON: Yes, certainly. There's another issue: when my learned friend interrupted yesterday, he said that something was going to happen, about which I have received no information.

MR JUSTICE GOSS: Oh yes.

MR JOHNSON: I want to sort that out before I finish as well.

MR MYERS: At this stage, there's nothing we -- I know the point my learned friend refers to, there's nothing we go to there at the moment, there's nothing we can go to there at the moment, for a number of obvious reasons.

As I recall, after I had raised that, the prosecution said, "The point's been made", as if the matter was at an end, so I don't understand where we go from there, but in any event that was the last thing Mr Johnson said on the matter when we raised it yesterday. There's nothing more we can do about that at this point.

As for the timing tomorrow, we're in your Lordship's hands whether it's 10.00 or 10.30. It's something the defendant is entitled to be present for when the matter is argued and 10.30 may be -- 10.00 may be more difficult than 10.30, I don't know, I'm in the court's hands.

MR JUSTICE GOSS: Given that no one will be required to see the defendant before 10 o'clock...

MR MYERS: No, apart from possibly Ms Santolini.

MR JUSTICE GOSS: Is she available before 10 o'clock?

DOCK OFFICER: (Inaudible: no microphone).

MR MYERS: Whatever time suits the court then, my Lord.

MR JUSTICE GOSS: Certainly. We'll say 10 o'clock then.

I don't want to inconvenience the jury unnecessarily having said 10.30 for them tomorrow.

MR MYERS: To be clear, as for that other matter, that's nothing we can deal with at this stage. Whether there is anything

arising from it or not, we shall see, but yes, I go back and check that the response from the prosecution in any event was, "The point has been made".

MR JUSTICE GOSS: I'll leave it to you to discuss with Mr Johnson to see whether either of you wish to take that matter any further on either side.

Can I say, in relation to the matter we will be addressing at 10 o'clock tomorrow, do you want to put anything in writing, Mr Myers, before we start?

You will have seen what Mr Johnson has said.

MR MYERS: Yes. I'll endeavour to do so. There's a lot we're dealing with --

MR JUSTICE GOSS: I know there is a lot.

MR MYERS: We'll endeavour to deal with that overnight if you can.

MR JUSTICE GOSS: Yes, if you can, with the assistance perhaps of your juniors, even if you aren't the scribe yourself or the typist.

MR MYERS: We'll endeavour to put something together, my Lord.

MR JUSTICE GOSS: It'll just save a bit of time in the morning.

MR JOHNSON: I know your Lordship's love for the paper version.

(Handed)

MR JUSTICE GOSS: I do have a paper version but not in colour, so that's of great assistance because I don't have access to a colour photocopier. Thank you very much.

Right, thank you very much. So the court will be sitting at 10 o'clock tomorrow but the jury will be here at 10.30.

I should have said, Mr Myers, but I didn't want to interrupt the court closing, so to speak: it needn't be a discursive document, bullet points are fine, it's just headline points rather than --

MR MYERS: We'll assist with that, my Lord.

MR JUSTICE GOSS: Thank you very much. Good. Thank you very much.

(4.20 pm)

(The court adjourned until 10.00 am on Friday, 9 June 2023)